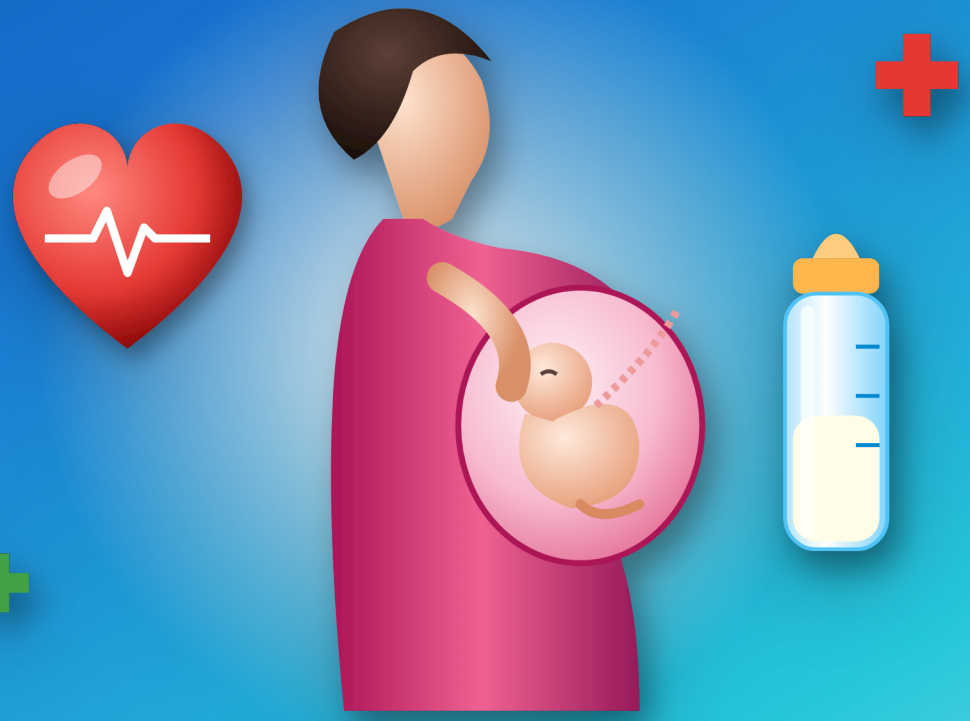
GSSSB · STAFF NURSE EXAM

Midwifery & OBG MCQ Bank

670 MCQs · GSSSB pattern · Answer key with explanation



20 Sections | 670 MCQs | Direct + Priority + Scenario | Match & Assertion-Reason | Calculations | Explanations

Hinglish explanation + English exam terms

Sister Asha
Aapki Nursing teacher

MCQ Bank – Sections

1. **Introduction, Terminology & MCH Programmes**
   30 MCQs · Obstetric terms, GTPAL, MMR/IMR, JSY, JSSK, PMSMA, LaQshya, SUMAN, Gujarat schemes, important days
2. **Anatomy, Pelvis & Fetal Skull**
   30 MCQs · Female reproductive organs, uterine supports, pelvis types & diameters, fetal skull sutures, fontanelles, diameters, moulding
3. **Menstruation, Fertilisation & Fetal Development**
   30 MCQs · Menstrual cycle hormones & phases, ovulation, fertilisation, implantation, embryo/ fetal milestones, teratogens
4. **Placenta, Membranes, Liquor & Fetal Circulation**
   30 MCQs · Placental hormones & functions, cord, abnormal placentas, amniotic fluid disorders, fetal circulation shunts
5. **Physiological Changes in Pregnancy**
   30 MCQs · Uterus, cervix, CVS, blood, respiratory, renal, GI, skin, endocrine changes; minor disorders
6. **Diagnosis of Pregnancy & Antenatal Care**
   30 MCQs · Signs of pregnancy, pregnancy tests, Naegele's rule, ANC schedule, IFA, Td, calcium, GDM screening, danger signs, diet
7. **Abdominal Examination & Fetal Wellbeing**
   30 MCQs · Fundal height, Leopold grips, lie/ presentation/ position, engagement, kick count, NST, BPP, CTG, Doppler, prenatal diagnosis
8. **Normal Labour**
   30 MCQs · Onset, true vs false labour, stages, mechanism, partograph, management of each stage, AMTSL, episiotomy, perineal tears
9. **Normal Puerperium & Breastfeeding**
   30 MCQs · Involution, lochia, PNC visits, postnatal care, lactation physiology, breastfeeding techniques, breast problems
10. **Newborn Care & Resuscitation**
    30 MCQs · ENBC, APGAR, NRP golden minute, thermal care, cord care, vitamin K, vaccines at birth, reflexes, jaundice, KMC, LBW
11. **High-Risk Pregnancy & Medical Disorders**
    30 MCQs · Hyperemesis, anaemia, PIH/ pre-eclampsia/ eclampsia & MgSO4, GDM, heart disease, Rh isoimmunisation, infections, twins, IUGR, IUD
12. **Bleeding in Pregnancy – Abortion, Ectopic, Mole, APH**
    30 MCQs · Types of abortion, MTP Act 2021, ectopic pregnancy, hydatidiform mole, placenta praevia vs abruption, vasa praevia
13. **Abnormal Labour & Malpresentations**
    30 MCQs · OP position, breech, face, brow, shoulder, CPD, obstructed labour, prolonged labour, preterm labour, PROM, post-term, induction, Bishop score

14 Obstetric Emergencies & PPH
30 MCQs · PPH 4Ts, management steps, retained placenta, inversion, rupture uterus, cord prolapse, shoulder dystocia, AFE, DIC, shock

15 Puerperal Complications & Neonatal Problems
30 MCQs · Puerperal sepsis, UTI, DVT, subinvolution, breast problems, postpartum psychiatric disorders, RDS, asphyxia, sepsis, birth injuries, anomalies

16 Obstetric Operations & Labour Analgesia
30 MCQs · LSCS vs classical CS, pre/ post-op care, forceps, vacuum, ARM, ECV, MRP, MVA, D&C, cerclage, analgesia

17 Drugs in Obstetrics
30 MCQs · Uterotonics, prostaglandins, tocolytics, MgSO4, antihypertensives, steroids, anti-D, anticoagulants, antibiotics, drugs in lactation

18 Family Planning & Contraception
30 MCQs · Pearl index, basket of choice, IUCD, PPIUCD, OCP (Mala-N), Chhaya, Antara, emergency contraception, LAM, barrier, sterilization

19 Gynaecological Disorders & Women's Health
30 MCQs · Menstrual disorders, PCOS, endometriosis, infertility, fibroid, prolapse, RTI/ STI, PID, cancers & screening, menopause

20 GSSSB Pattern Mock Test – 100 MCQs (Full OBG Syllabus)
100 MCQs · 100 questions, 100 marks, 0.25 negative marking – complete Midwifery & Obstetrics + Gynaecology

Level 1 – Direct: facts, values, definitions

Level 2 – Priority: "first / best / immediate" action

Level 3 – Advanced: scenario, NOT/EXCEPT, match, A-R, calculation



MCQ kaise solve karein?

Pehle khud answer socho, phir options padho. Har section ke baad answer key aur explanation hai, galti wale questions ka number apni notebook me likho. Exam me galat answer par **0.25 marks** kat sakte hain, isliye bina soche guess mat karo.

1 Introduction, Terminology & MCH Programmes

Level 1 – Direct

1. "Father of Obstetrics" kise kaha jata hai?
   (a) James Simpson (b) Naegele (c) William Smellie (d) Semmelweis
2. Puerperal fever rokne ke liye hand washing kisne shuru karaya?
   (a) Koch (b) Lister (c) Pasteur (d) Semmelweis
3. Maternal Mortality Ratio (MMR) kis base par express hota hai?
   (a) 1,00,000 women 15–49 yrs (b) 1000 population (c) 1000 live births (d) 1,00,000 live births
4. India me maternal death ka sabse common cause –
   (a) Abortion (b) Sepsis (c) PPH (d) Eclampsia
5. Infant Mortality Rate (IMR) kis per hota hai?
   (a) 1000 population (b) 1000 live births (c) 100 births (d) 1,00,000 live births
6. Janani Suraksha Yojana (JSY) kab launch hui?
   (a) 2000 (b) 2005 (c) 2011 (d) 2016
7. JSSK (Janani Shishu Suraksha Karyakram) kab shuru hua?
   (a) 2005 (b) 2008 (c) 2011 (d) 2014
8. PMSMA ke antargat free ANC checkup har mahine kis tarikh ko?
   (a) 1st (b) 9th (c) 15th (d) 21st
9. Primigravida ka matlab –
   (a) Kabhi pregnant na hui (b) Pehli baar pregnant hui woman (c) Jisne ek baar delivery ki ho (d) 5 deliveries wali
10. Grand multipara –
    (a) ≥ 3 deliveries (b) ≥ 4 deliveries (c) ≥ 5 deliveries (d) ≥ 7 deliveries
11. Nulligravida –
    (a) Kabhi pregnant nahi hui (b) Pehli pregnancy (c) Infertile (d) Kabhi viable baby nahi diya
12. National Safe Motherhood Day –
    (a) 7 April (b) 8 March (c) 11 April (d) 12 May
13. Puerperium ki duration –
    (a) 2 weeks (b) 4 weeks (c) 6 weeks (d) 12 weeks
14. LaQshya programme kisse related hai?
    (a) Family planning (b) Newborn vaccination (c) Labour room & maternity OT quality (d) Adolescent health
15. PMMVY me pehle bachche ke liye benefit –
    (a) ₹ 10,000 (b) ₹ 3,000 (c) ₹ 5,000 (d) ₹ 1,000
16. Viability ki gestational age (India, legal-obstetric) –
    (a) 20 wks (b) 24 wks (c) 28 wks (d) 32 wks
17. World Breastfeeding Week –
    (a) 1–7 August (b) 1–7 July (c) 14–21 November (d) 1–7 September

18. Gujarat ki Chiranjeevi Yojana kis cheez ke liye famous hai?
(a) Free spectacles (b) School health (c) Private doctors dwara BPL mothers ki free delivery (d) Free vaccination

? Level 2 – Priority & Application

19. Ek woman 3rd baar pregnant hai; ek term baby, ek abortion (10 wks), 1 living child hai. Uska GTPAL –
(a) G3 T1 P1 A0 L1 (b) G3 T2 P0 A1 L1 (c) G3 T1 P0 A1 L1 (d) G2 T1 P0 A1 L1

20. Twins ki ek delivery ko parity me kaise count karte hain?
(a) 0 (b) 1 (c) Gravida me 2 (d) 2

21. JSSK ke antargat pregnant woman ko kya free milta hai?
(a) Sirf cash (b) Sirf transport (c) Delivery, CS, drugs, diagnostics, diet, blood, transport (d) Sirf drugs

22. Ek woman 28 hafte ki pregnancy me labour me aati hai. Ye kaunsi delivery kahlayegi?
(a) Preterm (b) Term (c) Abortion (d) Post-term

23. SUMAN initiative ka main focus –
(a) Only TT vaccination (b) Only family planning (c) Only nutrition (d) Zero tolerance for preventable maternal & newborn deaths, assured free services

24. ASHA ka JSY me main role –
(a) Sterilization (b) Institutional delivery ke liye mother ko motivate aur escort karna (c) Drug likhna (d) CS karna

25. Pregnancy ke 42 din ke andar accident se mrityu – kya ye maternal death hai?
(a) Nahi – accidental/ incidental cause (b) Haan (c) Sirf hospital me ho to (d) Sirf CS ke baad

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Maternal death ki definition me kaunsi baat NAHI hai?
(a) Accidental causes include hote hain (b) Pregnancy ke dauran ya 42 din ke andar (c) Duration/ site of pregnancy irrespective (d) Pregnancy se related ya aggravated cause

27. Ek district me 1 saal me 40,000 live births aur 36 maternal deaths huye. MMR –
(a) 9 (b) 36 (c) 90 (d) 900

28. Match: P. JSY Q. JSSK R. PMSMA S. PMMVY — with 1. 9th of every month ANC 2. Cash for institutional delivery 3. Maternity benefit cash (wage loss) 4. Free entitlements incl. transport
(a) P-2, Q-1, R-4, S-3 (b) P-2, Q-4, R-1, S-3 (c) P-3, Q-4, R-1, S-2 (d) P-4, Q-2, R-1, S-3

29. Assertion (A): Institutional delivery maternal mortality kam karti hai. Reason (R): Skilled birth attendant PPH aur eclampsia jaisi emergencies turant manage kar sakta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

30. Kaun sa term SAHI match NAHI hai?
(a) Grand multipara – ≥ 5 deliveries (b) Multigravida – 2nd ya usse zyada pregnancy (c) Nullipara – kabhi pregnant nahi hui hi hogi (d) Primipara – ek baar viable baby deliver kiya

✓ Answer Key & Explanation

1-c William Smellie – Father of British midwifery/ obstetrics.

2-d Ignaz Semmelweis – chlorinated lime hand washing (1847).

3-d MMR = maternal deaths per 1,00,000 live births.
4-c PPH sabse common direct cause.
5-b IMR – per 1000 live births.
6-b JSY – 12 April 2005 (NRHM).
7-c JSSK – June 2011.
8-b PMSMA – har mahine 9 tarikh.
9-b Primigravida – first pregnancy.
10-c Grand multipara ≥ 5 deliveries.
11-a Nulligravida – kabhi pregnant nahi.
12-c 11 April (Kasturba Gandhi jayanti).
13-c Puerperium – 6 weeks.
14-c LaQshya (2017) – labour room quality.
15-c PMMVY – ₹ 5,000 pehle bachche ke liye.
16-c India me viability 28 wks (WHO/ new – 24 wks bhi likha jata hai; GSSSB me 28).
17-a 1–7 August.
18-c Chiranjeevi Yojana – private obstetricians ke saath PPP model.
19-c G3 (current incl.), T1, P0, A1, L1.
20-b Multiple pregnancy parity me 1 hi count hoti hai.
21-c JSSK – sab cashless entitlements.
22-a 28 wks – preterm (< 37 wks).
23-d SUMAN (2019) – zero preventable deaths.
24-b ASHA – motivator & escort.
25-a Accidental/ incidental cause maternal death nahi.
26-a Accidental causes exclude hote hain.
27-c (36 ÷ 40,000) × 1,00,000 = 90.
28-b JSY cash, JSSK entitlements, PMSMA 9th, PMMVY maternity benefit.
29-a Dono sahi, R reason hai.
30-c Nullipara – jisne kabhi viable baby deliver nahi kiya (pregnant ho chuki ho sakti hai).

2 Anatomy, Pelvis & Fetal Skull

? Level 1 – Direct

1. Non-pregnant uterus ka weight –
   (a) 20–30 g (b) 50–60 g (c) 100 g (d) 200 g
2. Fertilization ki normal site –
   (a) Uterine cavity (b) Ampulla (c) Isthmus (d) Infundibulum
3. Fallopian tube ki length –
   (a) 5 cm (b) 7.5 cm (c) 10 cm (d) 15 cm
4. Tubectomy (Pomeroy) kis part par hoti hai?
   (a) Isthmus (b) Interstitial (c) Ampulla (d) Fimbria
5. Uterus ka main support (Mackenrodt's ligament) –
   (a) Ovarian ligament (b) Round ligament (c) Cardinal ligament (d) Broad ligament
6. Sabse common pelvis type (female) –
   (a) Android (b) Anthropoid (c) Gynaecoid (d) Platypelloid
7. Pelvic inlet ka sabse bada diameter –
   (a) Transverse (b) Diagonal (c) Oblique (d) AP
8. Obstetric conjugate ki normal length –
   (a) 8 cm (b) 9 cm (c) 10–10.5 cm (d) 13 cm
9. Clinically PV se measure hone wala conjugate –
   (a) True conjugate (b) Diagonal conjugate (c) Anatomical conjugate (d) Obstetric conjugate
10. Anterior fontanelle (bregma) ka shape –
    (a) Diamond (b) Square (c) Triangle (d) Circle
11. Anterior fontanelle kab band hota hai?
    (a) 6 months (b) 3 years (c) 18 months (d) 6–8 weeks
12. Posterior fontanelle kab band hota hai?
    (a) Birth par (b) 1 year (c) 6 months (d) 6–8 weeks
13. Fully flexed vertex me engaging diameter –
    (a) Occipito-frontal 11.5 cm (b) Mento-vertical 14 cm (c) Suboccipito-bregmatic 9.5 cm (d) Submento-bregmatic 9.5 cm
14. Brow presentation me engaging diameter –
    (a) Mento-vertical (b) Submento-bregmatic (c) Suboccipito-bregmatic (d) Occipito-frontal
15. Dono parietal bones ke beech wala suture –
    (a) Frontal (b) Sagittal (c) Coronal (d) Lambdoid
16. Anthropoid pelvis me inlet –
    (a) Round (b) Kidney-shaped (transverse > AP) (c) Heart-shaped (d) AP > transverse (oval)
17. Bartholin's glands kahan hoti hain?
    (a) Urethra ke upar (b) Cervix (c) Labia majora ke posterior part me, vaginal opening ke dono taraf (d) Clitoris
18. Uterus ka normal position –
    (a) Axial (b) Anteverted & anteflexed (c) Retroflexed (d) Retroverted

Level 2 – Priority & Application

19. Delivery ke baad nurse baby ke sir par swelling dekhti hai jo suture line cross karti hai aur 2–3 din me khatam ho jati hai. Ye kya hai?
(a) Subgaleal haematoma (b) Caput succedaneum (c) Cephalhaematoma (d) Hydrocephalus

20. Cephalhaematoma ki pehchaan –
(a) Suture cross karta hai (b) Pitting hota hai (c) Suture line cross NAHI karta, kai hafte rehta hai (d) Birth par hi maximum

21. Android pelvis wali woman me labour me kya common hai?
(a) Deep transverse arrest / occipito-posterior (b) Face presentation always (c) Precipitate labour (d) Twins

22. PV exam me sacral promontory touch nahi hota. Iska matlab –
(a) Pelvis contracted (b) CPD definite (c) Diagonal conjugate adequate (> 11.5 cm) (d) Malpresentation

23. Newborn me depressed (sunken) anterior fontanelle kis taraf ishara karta hai?
(a) Dehydration (b) Hydrocephalus (c) Normal (d) ↑ ICP

24. Bulging anterior fontanelle (baby shant hai) –
(a) Dehydration (b) Normal (c) ↑ Intracranial pressure – meningitis/ hydrocephalus (d) Hypoglycaemia

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

25. Uterus ke supports me kaun sa NAHI hai?
(a) Uterosacral ligament (b) Broad ligament – main support (c) Cardinal ligament (d) Pubocervical ligament

26. Diagonal conjugate 12.5 cm hai. Obstetric conjugate approx –
(a) 9 cm (b) 11 cm (c) 12.5 cm (d) 14 cm

27. Match: P. Gynaecoid Q. Android R. Anthropoid S. Platypelloid — with 1. Heart-shaped inlet 2. Round inlet 3. Flat, kidney shaped 4. Long oval AP
(a) P-2, Q-1, R-4, S-3 (b) P-1, Q-2, R-3, S-4 (c) P-2, Q-4, R-1, S-3 (d) P-3, Q-1, R-4, S-2

28. Assertion (A): Moulding se fetal head ka engaging diameter kam ho jata hai. Reason (R): Newborn ki skull bones aapas me fused hoti hain.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Pelvic outlet ke diameters me SAHI kaunsa hai?
(a) AP 10, transverse 10 (b) AP 11, transverse 13 (c) AP 13 cm, transverse 11 cm (d) AP 12, transverse 12

30. Fetal skull diameter aur presentation ka galat jodi kaunsa?
(a) Occipito-frontal – deflexed (OP) (b) Submento-bregmatic – breech (c) Suboccipito-bregmatic – flexed vertex (d) Mento-vertical – brow

Answer Key & Explanation

1-b 50–60 g (term par ~1000 g).
2-b Ampulla – sabse wide part.
3-c 10 cm.
4-a Isthmus – narrow, straight part.
5-c Cardinal (transverse cervical) ligament main support.
6-c Gynaecoid ~50 %.
7-a Transverse 13 cm.
8-c 10–10.5 cm – shortest AP of inlet.
9-b Diagonal conjugate (12–12.5 cm) – minus 1.5–2 cm = obstetric.
10-a Diamond-shaped.
11-c 18 months.
12-d 6–8 weeks.
13-c 9.5 cm – smallest.
14-a Mento-vertical 13.5–14 cm – no engagement.
15-b Sagittal suture.
16-d Anthropoid – long AP oval.

17-c Posterolateral to vaginal orifice (4 & 8 o'clock).
18-b Anteverted, anteflexed.
19-b Caput – oedema, crosses sutures, disappears 24–72 hrs.
20-c Cephalhaematoma – subperiosteal, suture tak simit, weeks me jata hai.
21-a Android – narrow forepelvis → OP, deep transverse arrest.
22-c Promontory not reached = adequate AP diameter.
23-a Sunken – dehydration.
24-c Bulging – ↑ ICP.
25-b Broad ligament main support nahi – sirf peritoneal fold.
26-b 12.5 − 1.5 = 11 cm (approx).
27-a Gynaecoid round, android heart, anthropoid oval AP, platypelloid flat.
28-c A sahi; R galat – bones fused nahi hoti, isliye moulding possible.
29-c Outlet AP 13, transverse 11 cm.
30-b Submento-bregmatic – face presentation, breech nahi.

3 Menstruation, Fertilisation & Fetal Development

Level 1 – Direct

1. 28 din ke cycle me ovulation kis din hota hai?
   (a) Day 14 (b) Day 7 (c) Day 28 (d) Day 21
2. Ovulation se pehle kaunsa hormone surge karta hai?
   (a) FSH (b) Prolactin (c) Oxytocin (d) LH
3. Ovulation ke baad BBT kis hormone ke karan badhta hai?
   (a) Oestrogen (b) FSH (c) hCG (d) Progesterone
4. Proliferative phase kis hormone ke under hota hai?
   (a) Progesterone (b) Oestrogen (c) Prolactin (d) LH
5. Secretory (luteal) phase ki duration lagbhag constant hoti hai –
   (a) 7 days (b) 10 days (c) 14 days (d) 21 days
6. Corpus luteum pregnancy me kis hormone se maintain hota hai?
   (a) FSH (b) hPL (c) hCG (d) Oxytocin
7. Ovum ki life ovulation ke baad –
   (a) 5 days (b) 7 days (c) 12–24 hrs (d) 48–72 hrs
8. Sperm female genital tract me kitni der fertile rehta hai?
   (a) 6 hrs (b) 10 days (c) 12 hrs (d) 48–72 hrs (up to 5 days)
9. Implantation fertilisation ke kitne din baad?
   (a) Day 14 (b) Day 6–7 (c) Day 1–2 (d) Day 21
10. Implant hone wali stage –
    (a) Zygote (b) Embryo 8 weeks (c) Morula (d) Blastocyst
11. Organogenesis ka period (sabse zyada teratogen-sensitive) –
    (a) 0–2 wks (b) 3–8 wks (c) 12–20 wks (d) 28–40 wks
12. Fetal heart beat shuru hoti hai –
    (a) 12 wks (b) Day 40 (c) Day 22 (d) 20 wks
13. Embryo "fetus" kab kahlata hai?
    (a) 4 wks (b) 6 wks (c) 9 wks (8 wks completion ke baad) (d) 16 wks
14. Neural tube band hoti hai –
    (a) Day 10 (b) 12 wks (c) Day 28 (d) Day 60
15. Lanugo hair pure body par kab dikhte hain?
    (a) 8 wks (b) 12 wks (c) 20 wks (d) 38 wks
16. Surfactant production shuru hoti hai lagbhag –
    (a) 12 wks (b) 16 wks (c) 24 wks (d) 40 wks
17. Fetal sex external genitalia se kab pehchana ja sakta hai?
    (a) 6 wks (b) 8 wks (c) 12–14 wks (d) 28 wks

18. Thalidomide kis defect ke liye famous hai?
(a) Phocomelia (limb defect) (b) NTD (c) Cataract (d) Cleft lip

Level 2 – Priority & Application

19. Ek couple conceive karna chahta hai; regular 30-day cycle hai. Fertile ovulation day approx –
(a) Day 16 (b) Day 14 (c) Day 10 (d) Day 20

20. Folic acid NTD rokne ke liye kab shuru karna best hai?
(a) 2nd trimester (b) Preconception (pregnancy se pehle) (c) Quickening ke baad (d) Labour me

21. 1st trimester me ek woman ko German measles (rubella) hota hai. Baby me sabse common risk –
(a) Phocomelia (b) Teeth staining (c) Congenital cataract, deafness, PDA (d) Gray baby syndrome

22. Spinnbarkeit (stretchable clear cervical mucus) kis taraf ishara karta hai?
(a) Menstruation (b) Pregnancy (c) Ovulation (d) Menopause

23. Pregnant woman ko kaunsa vaccine contraindicated hai?
(a) MMR (live) (b) Inactivated influenza (c) Td (d) Hepatitis B

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

24. Teratogen aur defect ka galat jodi –
(a) Valproate – NTD (b) Tetracycline – teeth staining (c) Alcohol – phocomelia (d) Warfarin – nasal hypoplasia

25. Match: P. FSH Q. LH R. Progesterone S. Oestrogen — with 1. Follicle growth 2. Ovulation trigger 3. Secretory endometrium 4. Proliferative endometrium
(a) P-4, Q-2, R-3, S-1 (b) P-1, Q-3, R-2, S-4 (c) P-2, Q-1, R-4, S-3 (d) P-1, Q-2, R-3, S-4

26. Assertion (A): Menstruation progesterone withdrawal se hota hai. Reason (R): Pregnancy na hone par corpus luteum degenerate ho jata hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

27. 35 din ke cycle me ovulation approx kis din?
(a) Day 17 (b) Day 21 (c) Day 28 (d) Day 14

28. Haase's rule se 5 lunar months ke fetus ki length –
(a) 20 cm (b) 25 cm (c) 30 cm (d) 35 cm

29. Kaunsa statement SAHI NAHI hai?
(a) Fertilisation ampulla me hota hai (b) Implantation normally cervix me hota hai (c) Sex sperm decide karta hai (d) Zygote 46 chromosomes ka hota hai

30. Haase's rule – first 5 months me fetal length = ?
(a) Month + 5 (b) Month × 5 (c) Month × 10 (d) Month^2

Answer Key & Explanation

1-a Day 14 (next period se 14 din pehle).
2-d LH surge – 24–36 hrs baad ovulation.
3-d Progesterone thermogenic – 0.3–0.5 °C rise.
4-b Oestrogen – proliferative phase.
5-c Luteal phase ~14 days fixed.
6-c hCG corpus luteum ko 8–10 wks tak maintain karta hai.
7-c Ovum 12–24 hrs.
8-d Sperm 48–72 hrs (up to 5 days).
9-b Day 6–7.
10-d Blastocyst.
11-b 3–8 wks organogenesis.
12-c Day 22 (~5 wks LMP se).
13-c 8 wks complete hone ke baad fetus.

14-c Day 28 – isliye preconception folic acid.
15-c ~20 wks.
16-c ~24 wks (mature ~34–35 wks).
17-c 12–14 wks (USG).
18-a Thalidomide – phocomelia.
19-a 30 − 14 = day 16.
20-b Preconception (≥ 1 month pehle) se 12 wks tak.
21-c Congenital rubella syndrome – cataract, deafness, PDA.
22-c Ovulation – oestrogen peak, clear stretchy mucus.
23-a Live vaccines (MMR) pregnancy me contraindicated.
24-c Alcohol – fetal alcohol syndrome; phocomelia thalidomide se.
25-d Sahi physiology.
26-a Dono sahi, R reason hai.
27-b 35 − 14 = day 21.
28-b 5 × 5 = 25 cm.
29-b Implantation normally upper posterior uterine wall (fundus) par.
30-d First 5 months – month^2 (cm); baad me month × 5.

4 Placenta, Membranes, Liquor & Fetal Circulation

? Level 1 – Direct

1. Term placenta ka weight –
(a) 250 g (b) 500 g (c) 1000 g (d) 1500 g

2. Placenta weight : baby weight ratio –
(a) 1 : 6 (b) 1 : 2 (c) 1 : 10 (d) 1 : 4

3. Umbilical cord me vessels –
(a) 1 artery 1 vein (b) 2 veins 1 artery (c) 2 arteries 1 vein (d) 2 arteries 2 veins

4. Cord ki normal length –
(a) 20 cm (b) 50 cm (c) 80 cm (d) 100 cm

5. Cord ko surround karne wala jelly –
(a) Decidua (b) Amnion jelly (c) Wharton's jelly (d) Chorion

6. Pregnancy test kis hormone par based hai?
(a) Progesterone (b) hPL (c) Oestriol (d) hCG

7. Anti-insulin (diabetogenic) placental hormone –
(a) Relaxin (b) hCG (c) Oxytocin (d) hPL

8. Placenta cross karne wala ek matra immunoglobulin –
(a) IgA (b) IgG (c) IgE (d) IgM

9. Term par amniotic fluid ki matra –
(a) 200 ml (b) 800 ml (c) 1500 ml (d) 2500 ml

10. Polyhydramnios –
(a) > 500 ml (b) > 1000 ml (c) > 2000 ml (d) > 4000 ml

11. Oligohydramnios ka AFI –
(a) < 5 cm (b) < 10 cm (c) < 15 cm (d) < 25 cm

12. Nitrazine paper amniotic fluid se kis colour ka hota hai?
(a) Green (b) Red (c) Blue (d) Yellow

13. Fetus me sabse zyada oxygenated blood kahan hota hai?
(a) Aorta (b) Umbilical vein (c) Umbilical artery (d) Pulmonary artery

14. Ductus arteriosus kise jodta hai?
(a) Pulmonary artery – aorta (b) Umbilical vein – IVC (c) Aorta – IVC (d) Right atrium – left atrium

15. Foramen ovale birth ke baad kya ban jata hai?
(a) Fossa ovalis (b) Ligamentum arteriosum (c) Ligamentum teres (d) Ligamentum venosum

16. Ductus venosus ka remnant –
(a) Ligamentum venosum (b) Fossa ovalis (c) Medial umbilical ligament (d) Ligamentum teres

17. Velamentous insertion of cord me cord kahan judta hai?
(a) Membranes (b) Center (c) Fetal side (d) Edge

18. Battledore placenta –
(a) Extra lobe (b) Cord membranes par (c) Two placentas (d) Cord margin par

? Level 2 – Priority & Application

19. ARM ke baad liquor green (meconium stained) hai. Nurse ka first action –
(a) FHS check karke doctor ko inform karna (b) Documentation karke wait (c) Oxytocin badhana (d) Mother ko walk karana

20. Polyhydramnios wali mother me ARM ke turant baad sabse bada risk –
(a) Hypertension (b) Inversion (c) Cord prolapse & abruption (d) Infection

21. Oligohydramnios kis fetal anomaly se associated hai?
(a) Renal agenesis (Potter's) (b) Oesophageal atresia (c) Spina bifida (d) Anencephaly

22. Polyhydramnios kis anomaly se associated hai?
(a) Renal agenesis (b) IUGR (c) Oesophageal atresia (d) Posterior urethral valve

23. Placenta deliver hone ke baad check karna kyon zaroori hai?
(a) Baby sex (b) Blood group (c) Weight ke liye (d) Retained cotyledons/ succenturiate lobe – PPH & sepsis rokne

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

24. Placenta ka function NAHI hai –
(a) Nutritive (b) Excretory (c) Fetal RBC production (d) Respiratory

25. Single umbilical artery kis cheez ka indication hai?
(a) Twins (b) Rh incompatibility (c) Normal variant, kuch fetal anomalies (renal, cardiac) ka risk (d) Post-maturity

26. Match: P. Foramen ovale Q. Ductus arteriosus R. Ductus venosus S. Umbilical vein — with 1. Ligamentum arteriosum 2. Fossa ovalis 3. Ligamentum teres 4. Ligamentum venosum
(a) P-2, Q-1, R-4, S-3 (b) P-1, Q-2, R-3, S-4 (c) P-2, Q-4, R-1, S-3 (d) P-3, Q-1, R-4, S-2

27. Assertion (A): Birth ke baad foramen ovale functionally band ho jata hai. Reason (R): Pehli saans ke baad left atrium ka pressure right atrium se badh jata hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

28. Placenta accreta spectrum me SAHI kaunsa?
(a) Percreta – sirf decidua (b) Accreta – myometrium tak, Increta – myometrium ke andar, Percreta – serosa ke paar (c) Accreta – serosa ke paar (d) Teeno same

29. Amniotic fluid ka function NAHI hai –
(a) Shock absorber (b) Fetus ko oxygen supply (c) Temperature maintain (d) Fetal movement allow

30. AFI four quadrants: 4, 5, 6, 5 cm. Interpretation –
(a) Anhydramnios (b) Oligohydramnios (c) Polyhydramnios (d) Normal (20 cm)

✓ Answer Key & Explanation

1-b ~500 g.
2-a 1 : 6.
3-c 2 arteries (deoxygenated), 1 vein (oxygenated).
4-b ~50 cm.
5-c Wharton's jelly.
6-d hCG – peak 8–10 wks.
7-d hPL (human placental lactogen).
8-b IgG – passive immunity.

9-b ~800 ml (peak 1000 ml at 36–38 wks).
10-c > 2000 ml.
11-a AFI < 5 cm.
12-c Liquor alkaline → blue.
13-b Umbilical vein (~80 % saturation).
14-a Pulmonary artery → aorta (lungs bypass).
15-a Fossa ovalis.
16-a Ligamentum venosum.
17-a Membranes par – vasa praevia risk.
18-d Battledore – marginal insertion.
19-a Meconium – fetal distress possibility – FHS & inform.
20-c Sudden decompression – cord prolapse & abruption.
21-a Fetal urine nahi – oligohydramnios.
22-c Fetus fluid swallow nahi kar pata.
23-d Incomplete placenta – PPH/ sepsis.
24-c RBCs fetus khud banata hai (liver, spleen, marrow).
25-c SUA – associated anomalies ka risk.
26-a Sahi remnants.
27-a Dono sahi, R reason hai.
28-b Accreta < increta < percreta depth.
29-b Oxygen placenta deta hai, liquor nahi.
30-d 4+5+6+5 = 20 cm – normal (5–25).

5 Physiological Changes in Pregnancy

Level 1 – Direct

1. Pregnancy me blood volume kitna badhta hai?
   (a) 10 % (b) 20 % (c) 40–50 % (d) 100 %
2. Pregnancy me physiological anaemia ka karan –
   (a) Plasma volume RBC mass se zyada badhna (haemodilution) (b) Bleeding (c) Bone marrow suppression (d) Iron loss
3. Cardiac output maximum kab hota hai?
   (a) 20 wks (b) Delivery ke turant baad (c) 6 wks postpartum (d) 1st trimester
4. Supine hypotension syndrome ka karan –
   (a) Heart disease (b) Dehydration (c) Anaemia (d) Gravid uterus dwara IVC compression
5. Supine hypotension me best position –
   (a) Trendelenburg (b) Left lateral (c) Prone (d) Supine
6. Pregnancy me average weight gain (normal BMI) –
   (a) 5 kg (b) 10–12 kg (c) 20 kg (d) 25 kg
7. Chehre par brown pigmentation ("mask of pregnancy") –
   (a) Chloasma (b) Linea nigra (c) Striae (d) Spider naevi
8. Pregnancy me abdomen par stretch marks –
   (a) Striae gravidarum (b) Palmar erythema (c) Chloasma (d) Linea nigra
9. Pregnancy me ESR –
   (a) Kam (b) Badhta hai (c) Same (d) Zero
10. Pregnancy ek ___ state hai.
    (a) Normocoagulable (b) Hypercoagulable (c) Hypocoagulable (d) Haemorrhagic
11. Pregnancy me GFR –
    (a) Same (b) Kam hota hai (c) Zero (d) 50 % badhta hai
12. Pregnancy me constipation ka main karan –
    (a) Oxytocin (b) hCG (c) Progesterone – smooth muscle relaxation (d) Oestrogen
13. Morning sickness sabse zyada kab?
    (a) 6–12 wks (b) 20 wks (c) 32 wks (d) 38 wks
14. Heartburn pregnancy me kyon?
    (a) Gall stone (b) Cardiac sphincter relaxation (progesterone) + pressure (c) Ulcer (d) Infection
15. Pregnancy me uterus ka term weight –
    (a) 100 g (b) 500 g (c) 1000 g (d) 2000 g
16. Braxton Hicks contractions –
    (a) Abortion sign (b) Painless, irregular, no cervical change (c) Painful, regular, cervical dilatation (d) Only labour me
17. Pregnancy me respiratory change –
    (a) RR doubles (b) Respiratory acidosis (c) Tidal volume ↑, mild respiratory alkalosis (d) Tidal volume ↓

18. Pregnancy me thyroid –
(a) Hyperthyroid always (b) Mild enlargement, ↑ TBG (c) Shrink (d) No change

Level 2 – Priority & Application

19. 32 wks pregnant woman supine lete hue chakkar aur BP girne ki shikayat karti hai. Nurse kya kare?
(a) Doctor ko call karke wait (b) Legs raise karna (c) Oxygen mask (d) Left lateral turn karna

20. Morning sickness ke liye advice –
(a) Ek baar bhaari khana (b) Khaana skip karna (c) Uthne se pehle dry biscuit/ toast, small frequent meals (d) Fatty food

21. Pregnancy me leg cramps ke liye immediate relief –
(a) Foot dorsiflex karke calf stretch (b) Foot plantar flex karna (c) Ice (d) Walk fast

22. Pregnancy me varicose veins rokne ki advice –
(a) Tight garters (b) Bed rest only (c) Legs elevate, cross legs na karna, elastic stockings (d) Lambi der khade rehna

23. Pregnant woman frequent micturition 1st trimester me batati hai. Nurse –
(a) Reassure – uterus bladder par pressure; burning ho to report (b) UTI ka emergency treatment (c) Catheter (d) Fluids kam karne

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

24. Pregnancy me kaun sa change NORMAL NAHI hai?
(a) Glycosuria (mild) (b) Physiological dependent oedema (c) Linea nigra (d) Proteinuria > 300 mg/24 hr

25. Pregnancy me kya KAM hota hai?
(a) Serum albumin / Hb concentration (b) Fibrinogen (c) Plasma volume (d) Heart rate

26. Match: P. Hegar's sign Q. Goodell's sign R. Chadwick's sign S. Osiander's sign — with 1. Cervix softening 2. Isthmus softening 3. Bluish vagina 4. Lateral fornix pulsation
(a) P-2, Q-1, R-3, S-4 (b) P-2, Q-3, R-1, S-4 (c) P-1, Q-2, R-3, S-4 (d) P-4, Q-1, R-3, S-2

27. Assertion (A): Pregnancy me mother ko DVT ka risk badhta hai. Reason (R): Clotting factors ↑ aur venous stasis hota hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

28. Pre-pregnancy weight 50 kg, recommended gain 11 kg. Term par expected weight –
(a) 55 kg (b) 61 kg (c) 66 kg (d) 70 kg

29. Pregnancy me heart sounds ke liye kaun sa NORMAL nahi hai?
(a) Diastolic murmur (b) Third heart sound (c) Systolic ejection murmur (d) Heart rate 10–15 bpm ↑

30. Assertion (A): Pregnancy me UTI aur pyelonephritis zyada hote hain. Reason (R): Progesterone se ureters dilate hokar urinary stasis hota hai (right side zyada).
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

Answer Key & Explanation

1-c 40–50 % (~1.5 L).
2-a Haemodilution.
3-b Delivery ke turant baad (autotransfusion) – heart disease me critical.

4-d IVC compression → venous return ↓.
5-b Left lateral.
6-b 10–12 kg (~11 kg).
7-a Chloasma (melasma).
8-a Striae gravidarum.
9-b ESR ↑ – pregnancy me diagnostic value kam.
10-b Hypercoagulable – PPH se bachav, DVT risk.
11-d GFR ~50 % ↑.
12-c Progesterone – gut motility ↓.
13-a 6–12 wks (hCG peak).
14-b LES relaxation + uterus pressure.
15-c ~1000 g.
16-b Painless, irregular, no dilatation.
17-c Tidal volume ↑ – hyperventilation – alkalosis.
18-b Mild enlargement, TBG ↑.
19-d Left lateral – IVC se pressure hatao.
20-c Dry carbohydrate, small frequent meals.
21-a Dorsiflexion – calf stretch.
22-c Elevation, stockings.
23-a Physiological – reassure, UTI signs batao.
24-d Significant proteinuria – pre-eclampsia/ renal disease.
25-a Albumin & Hb concentration ↓ (dilution).
26-a Hegar isthmus, Goodell cervix, Chadwick bluish, Osiander pulsation.
27-a Dono sahi, R reason hai.
28-b 50 + 11 = 61 kg.
29-a Diastolic murmur hamesha pathological.
30-a Dono sahi, R reason hai.

6 Diagnosis of Pregnancy & Antenatal Care

Level 1 – Direct

1. Pregnancy ka pehla symptom –
   (a) Amenorrhoea (b) Morning sickness (c) Breast changes (d) Quickening
2. Pregnancy ka positive (certain) sign –
   (a) FHS sunai dena / USG me fetus (b) Positive urine hCG (c) Amenorrhoea (d) Hegar's sign
3. Quickening primigravida me kab mehsoos hoti hai?
   (a) 12 wks (b) 18–20 wks (c) 24 wks (d) 28 wks
4. Naegele's rule –
   (a) LMP + 10 months (b) LMP + 9 months + 7 days (c) LMP + 40 days (d) LMP + 9 months − 7 days
5. Pregnancy ki normal duration –
   (a) 260 days (b) 270 days (c) 280 days (d) 300 days
6. India (MoHFW) me minimum ANC visits –
   (a) 2 (b) 3 (c) 4 (d) 8
7. First ANC visit kab hona chahiye?
   (a) 12 wks ke andar (b) 20 wks (c) 28 wks (d) 36 wks
8. Pregnancy me IFA prophylaxis –
   (a) 60 mg iron + 500 µg folic acid daily × 180 days (b) 30 mg iron × 90 days (c) 100 mg iron × 30 days (d) Sirf folic acid
9. Pregnancy me Td vaccine ke doses (pehli pregnancy) –
   (a) 1 dose (b) 2 doses 4 weeks apart (c) 3 doses (d) 5 doses
10. Pregnancy me calcium supplementation –
    (a) 1 g/ day elemental (b) 2 g (c) 5 g (d) 500 mg
11. Albendazole pregnancy me kab diya jata hai?
    (a) 400 mg single dose 2nd trimester (b) 1st trimester (c) Never (d) Labour me
12. DIPSI test me glucose load –
    (a) 25 g (b) 50 g (c) 75 g (fasting/ non-fasting), 2 hr value (d) 100 g
13. DIPSI me GDM diagnosis –
    (a) 2 hr ≥ 140 mg/dl (b) ≥ 100 (c) ≥ 120 (d) ≥ 200
14. Urine pregnancy test (hCG) kitne din me positive ho jata hai?
    (a) 8 wks (b) 20 wks (c) 12 wks (d) Missed period ke aas-paas (~ 10–14 days after conception)
15. TVS se gestational sac kab dikhta hai?
    (a) 2 wks (b) 4.5–5 wks (c) 8 wks (d) 10 wks
16. Doppler se FHS kab sunai deta hai?
    (a) 6 wks (b) 10–12 wks (c) 20 wks (d) 24 wks
17. Fetal stethoscope (Pinard) se FHS –
    (a) 10 wks (b) 14 wks (c) 18–20 wks (d) 28 wks
18. MCP card ka full form –
    (a) Medical Child Program (b) Maternal Care Plan (c) Mother Care Pass (d) Mother & Child Protection card

Level 2 – Priority & Application

19. LMP 10 March 2026 hai. EDD –
(a) 10 December 2026 (b) 3 December 2026 (c) 17 December 2026 (d) 17 January 2027

20. ANC me kaun sa danger sign hai jiske liye turant hospital jaana chahiye?
(a) Frequent urination 1st trimester (b) Vaginal bleeding, severe headache, blurred vision, fits, reduced fetal movements (c) Heartburn (d) Halka backache

21. IFA tablet ke liye sahi advice –
(a) Empty stomach chai ke saath (b) Calcium ke saath ek saath (c) Chai/ doodh ke saath lena (d) Khane ke baad, vitamin C (nimbu) ke saath; kala stool normal hai

22. Pehli ANC visit me routine investigation NAHI hai –
(a) Blood group & Rh (b) Amniocentesis (c) HIV, VDRL, HBsAg (d) Hb

23. High-risk pregnancy ki pehchaan me kaun NAHI aata?
(a) Height < 145 cm (b) Age < 18 ya > 35 (c) Previous CS (d) 2nd gravida, age 25, Hb 12

24. Pregnant woman 12 wks par aati hai, uski pichli pregnancy 2 saal pehle thi aur Td ke 2 doses liye the. Ab kya?
(a) Kuch nahi (b) 2 doses (c) Td booster 1 dose (d) 3 doses

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

25. LMP 25 June. Naegele se EDD –
(a) 1 May (b) 2 April (c) 18 April (d) 25 March

26. Presumptive sign of pregnancy NAHI hai –
(a) Ballottement (b) Breast changes (c) Amenorrhoea (d) Nausea

27. Match: P. Presumptive Q. Probable R. Positive S. Pseudocyesis — with 1. FHS 2. Amenorrhoea 3. Hegar's/ ballottement 4. False belief of pregnancy
(a) P-2, Q-3, R-1, S-4 (b) P-3, Q-2, R-1, S-4 (c) P-1, Q-3, R-2, S-4 (d) P-2, Q-1, R-3, S-4

28. Assertion (A): Calcium supplementation pregnancy me di jati hai. Reason (R): Calcium pre-eclampsia ka risk kam karta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Pregnancy me extra calorie requirement (2nd–3rd trimester, ICMR) approx –
(a) +100 kcal (b) +350 kcal (c) +1000 kcal (d) +1500 kcal

30. Assertion (A): Ultrasound gestational age ke liye 1st trimester me sabse accurate hai. Reason (R): Crown-rump length 1st trimester me sabse reliable measure hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

Answer Key & Explanation

1-a Amenorrhoea – earliest symptom.
2-a FHS/ USG fetus/ fetal movements felt by examiner – positive signs.
3-b Primi 18–20 wks, multi 16–18 wks.
4-b LMP + 9 m + 7 d.
5-c 280 days (40 wks).
6-c Minimum 4 ANC (WHO 2016 – 8 contacts).
7-a 12 wks ke andar (early registration).
8-a 60 mg + 500 µg × 180 days (Anaemia Mukt Bharat).
9-b Td1 jaldi, Td2 4 wks baad.
10-a 1 g elemental calcium/ day.
11-a 400 mg single dose, 2nd trimester.

12-c 75 g oral glucose, 2 hr.
13-a ≥ 140 mg/dl.
14-d Missed period ke aas-paas.
15-b 4.5–5 wks TVS.
16-b 10–12 wks.
17-c 18–20 wks.
18-d Mother & Child Protection card.
19-c 10 March + 7 = 17 March, + 9 months = 17 December 2026.
20-b Ye sab danger signs – turant referral.
21-d Khane ke baad, vit C se absorption ↑; chai/ calcium se ↓.
22-b Amniocentesis routine nahi – sirf indication par.
23-d Ye normal low-risk hai.
24-c 3 saal ke andar 2 doses liye ho to 1 booster.
25-b 25 June + 7 = 2 July, + 9 m = 2 April.
26-a Ballottement – probable sign.
27-a Sahi.
28-a Dono sahi, R reason hai.
29-b ~ +350 kcal.
30-a Dono sahi, R reason hai.

7 Abdominal Examination & Fetal Wellbeing

Level 1 – Direct

1. Fundus umbilicus ke level par kab hota hai?
   (a) 12 wks (b) 16 wks (c) 22–24 wks (d) 36 wks
2. Fundus xiphisternum tak kab pahunchta hai?
   (a) 28 wks (b) 32 wks (c) 36 wks (d) 40 wks
3. Fundus just above symphysis pubis palpable hota hai –
   (a) 8 wks (b) 12 wks (c) 20 wks (d) 24 wks
4. Abdominal exam se pehle mother ko –
   (a) Bhookha rakhna (b) Bladder empty karwana (c) Sedation (d) Enema
5. Leopold ki 1st grip (fundal) se kya pata chalta hai?
   (a) Cephalic prominence (b) Back ka side (c) Fundus me kaun sa pole (head/ breech) (d) Engagement
6. Pawlik's grip (3rd) kis liye?
   (a) Liquor (b) Back (c) Fundal pole (d) Presenting part & uski mobility
7. Leopold ki 4th (pelvic) grip me examiner ka face –
   (a) Mother ke feet ki taraf (b) Mother ke face ki taraf (c) Back par (d) Side me
8. Sabse common lie –
   (a) Oblique (b) Unstable (c) Longitudinal (d) Transverse
9. Sabse common presentation –
   (a) Shoulder (b) Face (c) Cephalic (vertex) (d) Breech
10. Sabse common position –
    (a) LOA (b) LOP (c) ROP (d) ROA
11. Vertex presentation ka denominator –
    (a) Sacrum (b) Occiput (c) Acromion (d) Mentum
12. Engagement ka matlab –
    (a) Head perineum par (b) Head pelvis me nahi (c) Head ka biggest transverse diameter (BPD) inlet se pass ho gaya (d) Delivery
13. Normal FHR –
    (a) 60–80 bpm (b) 80–100 bpm (c) 110–160 bpm (d) 160–200 bpm
14. Fetal kick count (Cardiff "count to 10") – normal kam se kam –
    (a) 1 in 24 hrs (b) 3 in 12 hrs (c) 10 in 12 hrs (d) 20 in 1 hr
15. Reactive NST –
    (a) 20 min me 2 accelerations ≥ 15 bpm × 15 sec (b) FHR 100 (c) Late decelerations (d) 1 acceleration in 60 min
16. BPP (Biophysical profile) me kitne parameters?
    (a) 3 (b) 4 (c) 5 (d) 6
17. BPP ka sabse pehle effect hone wala (acute hypoxia) parameter –
    (a) Fetal tone (b) NST reactivity (c) Liquor (d) Breathing

18. Amniocentesis genetic diagnosis ke liye kab?
(a) 8 wks (b) 15–20 wks (c) 28 wks (d) 36 wks

? Level 2 – Priority & Application

19. CTG par late decelerations dikhte hain. Nurse ka first action –
(a) Mother ko left lateral turn, oxytocin band, O_2, IV fluids, doctor inform (b) Mother ko walk (c) Oxytocin badhana (d) Ignore

20. Variable decelerations ka common karan –
(a) Head compression (b) Normal sleep (c) Cord compression (d) Placental insufficiency

21. Early decelerations –
(a) Uteroplacental insufficiency (b) Cord prolapse (c) Fetal death (d) Head compression – benign

22. Mother kehti hai "aaj baby kam hil raha hai" (34 wks). Nurse –
(a) Kal aane ko kehna (b) Thanda pani pine ko kehna aur ghar bhejna (c) Turant FHS/ NST karwana (d) Sleep karne ko

23. Abdominal exam me fundus me hard round ballotable mass aur pelvis me soft broad part. Presentation –
(a) Face (b) Transverse (c) Cephalic (d) Breech

24. Fifth palpable (Crichton) method me head 2/5 abdomen me palpable hai. Matlab –
(a) Not engaged (b) Engaged (c) Delivered (d) Floating

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

25. VEAL CHOP me SAHI jodi –
(a) Variable – placenta (b) Early – cord (c) Variable – cord compression; Early – head compression; Accelerations – OK; Late – placental insufficiency (d) Late – head compression

26. Symphysio-fundal height 30 cm aur fetal gestation 30 wks. Interpretation –
(a) Twins (b) Polyhydramnios (c) IUGR (d) Size = dates (normal)

27. Match: P. CVS Q. Amniocentesis R. Cordocentesis S. Anomaly scan — with 1. 18–20 wks 2. 10–13 wks 3. 15–20 wks 4. After 18 wks fetal blood
(a) P-1, Q-3, R-4, S-2 (b) P-3, Q-2, R-4, S-1 (c) P-2, Q-3, R-4, S-1 (d) P-2, Q-4, R-3, S-1

28. Assertion (A): Fundal height dates se zyada hone par twins/ polyhydramnios sochte hain. Reason (R): Fundal height uterine contents ke size ko reflect karti hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Fundal height dates se KAM hone ka cause NAHI hai –
(a) Wrong dates (b) Oligohydramnios (c) IUGR (d) Macrosomia

30. BPP score 8/10 with normal liquor –
(a) Deliver immediately (b) Normal – low risk (c) Fetal death (d) Repeat in 1 hr always

✓ Answer Key & Explanation

1-c 22–24 wks.
2-c 36 wks (40 wks par lightening se thoda neeche).
3-b 12 wks.
4-b Bladder empty – accurate palpation.
5-c Fundal grip – fundus ka pole.
6-d Pawlik's – presenting part, engagement.
7-a Feet ki taraf face karke.
8-c Longitudinal ~99 %.

9-c Vertex ~96 %.
10-a LOA.
11-b Occiput.
12-c BPD through inlet.
13-c 110–160 bpm.
14-c 10 movements in 12 hrs.
15-a 2 accelerations 15 × 15 in 20 min.
16-c 5 (NST, breathing, movement, tone, liquor).
17-b NST reactivity sabse pehle jati hai; tone sabse last.
18-b 15–20 wks.
19-a Intrauterine resuscitation.
20-c Cord compression.
21-d Head compression – benign.
22-c Reduced movements – turant assessment.
23-d Fundus me head – breech presentation.
24-b ≤ 2/5 palpable – engaged.
25-c VEAL CHOP sahi mapping.
26-d 20–36 wks me SFH (cm) ≈ gestation ± 2.
27-c CVS 10–13, amnio 15–20, cordocentesis > 18, anomaly 18–20 wks.
28-a Dono sahi, R reason hai.
29-d Macrosomia me fundal height zyada hoti hai.
30-b 8–10 normal.

8 Normal Labour

Level 1 – Direct

1. True labour ka sabse reliable sign –
   (a) Progressive cervical dilatation & effacement (b) Show (c) Backache (d) Braxton Hicks
2. 1st stage of labour –
   (a) Onset of true labour se full dilatation tak (b) Baby birth se placenta (c) Full dilatation se baby birth (d) Placenta ke 1 hr baad
3. 2nd stage of labour –
   (a) Baby se placenta (b) Onset se full dilatation (c) Full dilatation se baby delivery (d) 1 hr after placenta
4. 3rd stage ki normal duration –
   (a) 1 hr (b) 2 hrs (c) 5–15 min (up to 30 min) (d) 6 hrs
5. 4th stage –
   (a) 6 hrs (b) Placenta ke 1 hr baad tak observation (c) 6 wks (d) 24 hrs
6. Primigravida me 1st stage ki average duration –
   (a) 2 hrs (b) 6 hrs (c) 12 hrs (d) 24 hrs
7. Labour ke mechanism me pehla step –
   (a) Internal rotation (b) Engagement (c) Descent (d) Flexion
8. Internal rotation kahan hota hai?
   (a) Inlet par (b) Pelvic floor par (c) Outlet ke baad (d) Uterus me
9. Head ka crowning kise kehte hain?
   (a) Restitution (b) Head contraction ke beech vulva se peeche nahi jata (c) Engagement (d) Head inlet par
10. Active phase of labour WHO (2018) me kab se?
    (a) 2 cm (b) 4 cm (c) 5 cm (d) 8 cm
11. Partograph me alert line ka rate –
    (a) 0.5 cm/hr (b) 1 cm/hr (c) 2 cm/hr (d) 3 cm/hr
12. Action line alert line se kitne ghante right?
    (a) 1 hr (b) 2 hrs (c) 4 hrs (d) 6 hrs
13. Partograph me descent kis se record hota hai?
    (a) Fundal height se (b) USG se (c) Abdominal fifths palpable "O" se (d) PV station se
14. 1st stage me FHS kitni der me?
    (a) Every 4 hrs (b) Every 2 hrs (c) Every 5 min (d) Every 30 min
15. 2nd stage me FHS –
    (a) Every 5 min / har contraction ke baad (b) Every 30 min (c) Every hr (d) Ek baar
16. PV exam labour me normally kitne ghante par?
    (a) 1 hr (b) 2 hrs (c) 4 hrs (d) 8 hrs
17. AMTSL me oxytocin ka dose –
    (a) 0.2 mg IM (b) 5 IU IV bolus (c) 10 IU IM baby birth ke 1 min ke andar (d) 20 IU oral
18. Most common episiotomy (India) –
    (a) J-shaped (b) Midline (c) Mediolateral (d) Lateral

Level 2 – Priority & Application

19. Placental separation ka sign NAHI hai –
(a) Gush of blood (b) Cord lengthening (c) Uterus soft aur flabby (d) Uterus globular, firm, rises

20. Partograph par cervical dilatation alert line ke right cross kar jata hai. Nurse –
(a) Mother ko ghar bhejna (b) Kuch nahi (c) Oxytocin bina order (d) Doctor ko inform, referral/ intervention ki taiyari

21. Delivery ke baad cord clamping (term, vigorous baby) –
(a) Turant (b) Placenta ke baad (c) 10 min baad (d) 1–3 min ke baad (delayed)

22. 2nd stage me mother ko kab push karna chahiye?
(a) Contraction ke beech (b) Har waqt (c) 4 cm par (d) Full dilatation aur urge aane par, contraction ke saath

23. Controlled cord traction ke dauran doosre haath se –
(a) Fundal pressure (b) Kuch nahi (c) Cord cut (d) Suprapubic counter-traction (Brandt-Andrews) – uterus ko upar dhakelna

24. 4th stage me nurse sabse pehle kya dekhti hai?
(a) Baby bath (b) Discharge (c) Diet (d) Uterine tone & bleeding, vitals every 15 min

25. Labour me bladder kyon empty rakhte hain?
(a) Full bladder descent rokta hai aur PPH/ injury ka risk badhata hai (b) Infection (c) Rule only (d) Comfort only

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Mechanism of labour ka sahi order –
(a) Engagement, descent, flexion, internal rotation, extension, restitution, external rotation, expulsion (b) Flexion, engagement, extension, rotation (c) Engagement, extension, flexion, restitution (d) Descent, engagement, extension, flexion

27. Perineal tear match: P. 1st degree Q. 2nd degree R. 3rd degree S. 4th degree — with 1. Rectal mucosa 2. Perineal muscles 3. Skin & vaginal mucosa 4. Anal sphincter
(a) P-2, Q-3, R-4, S-1 (b) P-3, Q-4, R-2, S-1 (c) P-1, Q-2, R-4, S-3 (d) P-3, Q-2, R-4, S-1

28. Assertion (A): Mediolateral episiotomy India me prefer ki jati hai. Reason (R): Isme anal sphincter tak extension ka risk kam hota hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Admission par 4 cm dilatation 8 AM. 12 PM par expected (alert line) dilatation –
(a) 5 cm (b) 6 cm (c) 8 cm (d) 10 cm

30. AMTSL ka component NAHI hai –
(a) Controlled cord traction (b) Fundal pressure for placenta delivery (c) Uterine massage after placenta (d) Uterotonic within 1 min

Answer Key & Explanation

1-a Cervical dilatation – true labour ka hallmark.
2-a Onset to full (10 cm) dilatation.
3-c Full dilatation to baby birth.
4-c 5–15 min; > 30 min retained placenta.
5-b 1 hr observation (PPH ka risk).
6-c Primi ~12 hrs, multi ~6 hrs.
7-b Engagement.
8-b Pelvic floor (levator ani) par.
9-b Crowning – head peeche nahi jata.
10-c WHO 2018 – 5 cm (LCG); India partograph – 4 cm.

11-b 1 cm/hr.
12-c 4 hrs.
13-c Fifths palpable "O".
14-d Every 30 min.
15-a Every 5 min.
16-c Every 4 hrs.
17-c 10 IU IM within 1 min.
18-c Mediolateral.
19-c Separation par uterus firm & globular hota hai.
20-d Alert line cross – inform, prepare referral.
21-d Delayed cord clamping 1–3 min.
22-d Full dilatation + urge.
23-d Brandt-Andrews – counter-traction (inversion se bachav).
24-d Tone, bleeding, vitals.
25-a Full bladder – descent rokta, atony.
26-a Standard order.
27-d 1st skin/ mucosa, 2nd muscles, 3rd sphincter, 4th rectal mucosa.
28-a Dono sahi, R reason hai.
29-c 4 cm + 4 hrs × 1 cm = 8 cm.
30-b Fundal pressure AMTSL ka part nahi – inversion risk.

9 Normal Puerperium & Breastfeeding

Level 1 – Direct

1. Delivery ke turant baad fundus ka level –
   (a) Not palpable (b) Umbilicus se thoda neeche (c) Xiphisternum (d) Symphysis
2. Uterus abdomen se palpable nahi rehta –
   (a) 3 days (b) 4 wks (c) 6 wks (d) 10–14 days
3. Fundus roz kitna neeche aata hai?
   (a) 0.5 cm (b) 1 cm (1 finger) (c) 3 cm (d) 5 cm
4. Lochia rubra ki duration –
   (a) 1–4 days (b) 3 wks (c) 5–9 days (d) 10–14 days
5. Lochia serosa ka colour –
   (a) Pinkish-brown (yellowish) (b) Green (c) Red (d) White
6. Lochia alba –
   (a) Day 10 – 6 wks (b) Only 1 day (c) Day 5–9 (d) Day 1–4
7. Afterpains kis me zyada hote hain?
   (a) Twins only (b) Primipara (c) Multipara & breastfeeding ke waqt (d) CS
8. Milk production (secretion) ka hormone –
   (a) Oestrogen (b) Oxytocin (c) Progesterone (d) Prolactin
9. Milk ejection (let-down) reflex ka hormone –
   (a) Insulin (b) Oxytocin (c) Prolactin (d) FSH
10. Colostrum me sabse zyada –
    (a) Protein & IgA (b) Fat (c) Iron (d) Lactose
11. Breastfeeding kab shuru karna chahiye (normal delivery)?
    (a) 1 hr ke andar (b) 3 days (c) 6 hrs (d) 24 hrs
12. Exclusive breastfeeding kitne mahine?
    (a) 3 (b) 4 (c) 6 (d) 12
13. Postnatal visits ka schedule (home based – India) me pehla visit –
    (a) Day 28 (b) Day 42 (c) Day 14 (d) Day 1/3
14. Puerperal pyrexia –
    (a) Any fever (b) ≥ 40 °C (c) ≥ 38 °C (100.4 °F) delivery ke 24 hr baad se 10 days (d) ≥ 37 °C
15. Postpartum blues kab common hai?
    (a) Day 1 (b) 3 months (c) Day 3–5 (d) 1 year
16. Foremilk me zyada –
    (a) Water & lactose (b) Protein only (c) Iron (d) Fat
17. Hindmilk me zyada –
    (a) Antibodies only (b) Water (c) Lactose only (d) Fat
18. Breast milk ki calorie approx –
    (a) 20 kcal/ 100 ml (b) 67–70 kcal/ 100 ml (c) 150 kcal/ 100 ml (d) 200 kcal

Level 2 – Priority & Application

19. Delivery ke 2 hrs baad fundus umbilicus se upar aur right side shift. Sabse probable cause –
(a) Retained placenta (b) Full bladder (c) Uterine rupture (d) Normal

20. Postnatal mother ka uterus soft (boggy) mila. First action –
(a) Ice (b) Fundal massage (c) Doctor ko call karke wait (d) Ambulation

21. Good attachment ka sign –
(a) Sirf nipple mouth me (b) Cheeks andar (c) Click sound (d) Baby ka chin breast ko touch, mouth wide open, lower lip outward, areola upar zyada dikhe

22. Breast engorgement me advice –
(a) Feeding band karna (b) Fluid restriction (c) Tight binder (d) Frequent feeding, warm compress, express milk

23. Mastitis me breastfeeding –
(a) Band (b) Sirf formula (c) 1 hafta band (d) Continue karna (affected side se bhi)

24. Sore/ cracked nipples ka sabse common cause –
(a) Baby teeth (b) Small nipple (c) Poor attachment (d) Infection

25. Breast abscess me –
(a) Sirf massage (b) Ignore (c) Incision & drainage, antibiotics; healthy side se feeding continue (d) Steroid

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Breastfeeding ke benefits me NAHI hai –
(a) PPH kam (b) LAM contraception (c) Mother me osteoporosis badhta hai long term (d) Breast/ ovarian cancer kam

27. Match: P. Rubra Q. Serosa R. Alba S. Lochiometra — with 1. Retention of lochia in uterus 2. Day 10–6 wk 3. Day 1–4 4. Day 5–9
(a) P-1, Q-4, R-2, S-3 (b) P-3, Q-4, R-2, S-1 (c) P-3, Q-2, R-4, S-1 (d) P-4, Q-3, R-2, S-1

28. Assertion (A): Baby ko breast se suckle karane se PPH kam hota hai. Reason (R): Suckling se oxytocin release hota hai jo uterus contract karta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Foul-smelling lochia kis taraf ishara karta hai?
(a) Normal (b) Puerperal infection (c) Lactation failure (d) Anaemia

30. Expressed breast milk room temperature par kitni der safe hai (approx)?
(a) 1 week (b) 3 days (c) 6–8 hrs (d) 30 min

Answer Key & Explanation

1-b Umbilicus se thoda neeche, firm.
2-d 10–14 days.
3-b ~1 cm/ day.
4-a Day 1–4.
5-a Pinkish-brown/ yellowish.
6-a Day 10 – 6 wks.
7-c Multipara, feeding par oxytocin.
8-d Prolactin.
9-b Oxytocin.
10-a Protein, IgA, vitamin A – "first vaccine".
11-a 1 hr ke andar (CS – jitni jaldi possible).
12-c 6 months.
13-d Day 1 (institutional) / 3 – phir 7, 14, 21, 28, 42.
14-c ≥ 38 °C after 24 hrs within 10 days.
15-c Day 3–5, 2 hafte me thik.
16-a Foremilk – watery, lactose.
17-d Hindmilk – fat rich.
18-b ~67–70 kcal/ 100 ml.

19-b Full bladder uterus ko upar & side me dhakelta hai.
20-b Fundal massage – contraction.
21-d Ye sab good attachment ke signs.
22-d Frequent emptying.
23-d Continue – stasis kam.
24-c Poor attachment.
25-c I&D + antibiotics.
26-c Breastfeeding bone health par long-term negative effect nahi dalta.
27-b Sahi.
28-a Dono sahi, R reason hai.
29-b Foul lochia – infection.
30-c Room temp ~6–8 hrs; fridge 24 hrs (approx).

10 Newborn Care & Resuscitation

Level 1 – Direct

1. APGAR score kab assess karte hain?
   (a) 1 & 5 min (b) Sirf 1 min (c) Birth & 1 hr (d) 10 & 20 min
2. APGAR me kitne parameters?
   (a) 3 (b) 4 (c) 5 (d) 6
3. APGAR 7–10 –
   (a) Dead (b) Moderate asphyxia (c) Normal (d) Severe asphyxia
4. Newborn me sabse pehle kya karna chahiye (birth ke turant baad)?
   (a) Dry & warm (b) Bath (c) Vaccination (d) Weight
5. "Golden minute" ka matlab –
   (a) Breastfeeding 1 min me (b) Cord cut (c) 1 min ke andar breathing establish/ PPV shuru (d) Vitamin K
6. Neonatal resuscitation me chest compression : ventilation –
   (a) 3 : 1 (b) 5 : 1 (c) 15 : 2 (d) 30 : 2
7. PPV ka rate –
   (a) 20/min (b) 40–60/min (c) 100/min (d) 120/min
8. Chest compression kab shuru karte hain?
   (a) Apnoea only (b) HR < 100 (c) HR < 120 (d) HR < 60 after 30 sec effective PPV
9. Newborn ko Vitamin K dose (term) –
   (a) 0.5 mg (b) 1 mg IM (c) 5 mg (d) 10 mg
10. Vitamin K kaunsi disease rokta hai?
    (a) Anaemia (b) Haemorrhagic disease of newborn (c) Jaundice (d) Rickets
11. Birth par diye jane wale vaccines –
    (a) DPT, measles (b) MR (c) Pentavalent (d) BCG, OPV-0, Hep B birth dose
12. Normal newborn heart rate –
    (a) 60–80 (b) 80–100 (c) 120–160 (d) 180–200
13. Normal newborn respiratory rate –
    (a) 10 (b) 12–20 (c) 30–60 (d) 70–90
14. LBW –
    (a) < 1500 g (b) < 2000 g (c) < 2500 g (d) < 3000 g
15. Moro reflex kab tak gayab ho jata hai?
    (a) 1 month (b) 1 year (c) 3–6 months (d) 2 years
16. Babinski reflex newborn me –
    (a) Abnormal (b) Flexor only (c) Absent (d) Positive (extensor) – normal
17. Physiological jaundice kab dikhta hai?
    (a) After 2 weeks (b) Day 1 only (c) After 24 hrs, peak day 3–5 (d) First 24 hrs
18. Newborn ko bath kab dena chahiye?
    (a) Turant (b) Cord girne ke baad hi (c) 24 hrs ke baad (term, stable) (d) 7 days baad

Level 2 – Priority & Application

19. Birth ke baad baby ro nahi raha, breathing nahi. Dry & stimulate ke baad bhi apnoea. Next step –
(a) Chest compression (b) Bag & mask PPV (room air) (c) Bath (d) Adrenaline

20. Pathological jaundice ka sign –
(a) Day 3 par aana (b) Day 5 me khatam (c) Sirf face par (d) First 24 hrs me aana

21. Phototherapy ke dauran nurse ka important care –
(a) Feeding band (b) Oil lagana (c) Baby ko kapde pehnana (d) Eyes & genitals cover, frequent feeding, temperature & hydration monitor, position change

22. Cord care ki current recommendation –
(a) Haldi/ ghee (b) Tight bandage (c) Kuch na lagana – dry cord care (chlorhexidine high NMR areas) (d) Cow dung

23. LBW baby ke liye best thermal care method (stable) –
(a) Fan (b) Kangaroo Mother Care (c) Bathing (d) Hot water bottle

24. Newborn me hypothermia –
(a) < 35 °C only (b) < 36.5 °C (c) < 37.5 °C (d) < 38 °C

25. Asymmetric Moro reflex kis taraf ishara karta hai?
(a) Jaundice (b) Normal (c) Hydrocephalus (d) Erb's palsy/ clavicle fracture

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Baby: HR 90, weak cry, some flexion, grimace, body pink extremities blue. APGAR –
(a) 4 (b) 5 (c) 6 (d) 7

27. Match: P. Rooting Q. Moro R. Palmar grasp S. Stepping — with 1. Startle on sudden movement 2. Cheek touch – head turns 3. Fingers grasp object 4. Walking movement when held upright
(a) P-4, Q-1, R-3, S-2 (b) P-2, Q-1, R-3, S-4 (c) P-1, Q-2, R-3, S-4 (d) P-2, Q-3, R-1, S-4

28. Assertion (A): Newborn ko hypothermia ka zyada risk hota hai. Reason (R): Newborn ka body surface area : weight ratio zyada aur subcutaneous fat kam hota hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Newborn ke liye kaun NORMAL NAHI hai?
(a) Mongolian spots (b) Milia (c) Central cyanosis (d) Physiological weight loss up to 10 % first week

30. KMC ka component NAHI hai –
(a) Skin-to-skin contact (b) Early discharge with follow-up (c) Exclusive breastfeeding (d) Incubator me rakhna

Answer Key & Explanation

1-a 1 & 5 min (low ho to 10 min tak har 5 min).
2-c 5 – Appearance, Pulse, Grimace, Activity, Respiration.
3-c 7–10 normal.
4-a Dry & warm – hypothermia prevention.
5-c Golden minute – 60 sec me PPV.
6-a 3 : 1 (90 compressions + 30 breaths/min).
7-b 40–60 breaths/min.
8-d HR < 60 after 30 sec effective PPV.
9-b 1 mg IM (preterm 0.5 mg).
10-b HDN – vitamin K deficiency bleeding.
11-d BCG, OPV-0, Hep B birth dose.
12-c 120–160.
13-c 30–60.
14-c < 2500 g.
15-c 3–6 months.
16-d Up to ~1–2 yrs extensor normal.
17-c After 24 hrs, peak 3–5 days.
18-c Bath delay ≥ 24 hrs.
19-b PPV with bag & mask.

20-d First 24 hrs – pathological.
21-d Ye sab phototherapy care.
22-c Dry cord care.
23-b KMC.
24-b < 36.5 °C.
25-d Erb's/ clavicle fracture.
26-b HR < 100 = 1, weak cry = 1, some flexion = 1, grimace = 1, acrocyanosis = 1 → 5.
27-b Sahi.
28-a Dono sahi, R reason hai.
29-c Central cyanosis – abnormal (acrocyanosis normal).
30-d KMC me mother hi "incubator" hai.

11 High-Risk Pregnancy & Medical Disorders

Level 1 – Direct

1. Pregnancy me anaemia (WHO) –
   (a) Hb < 12 (b) Hb < 10 (c) Hb < 7 (d) Hb < 11 g/dL
2. Pregnancy me anaemia ka sabse common cause –
   (a) B12 (b) Folate deficiency (c) Sickle cell (d) Iron deficiency
3. Pre-eclampsia –
   (a) Sirf oedema (b) Fits + HTN (c) 20 wks ke baad HTN + proteinuria (d) 20 wks se pehle HTN
4. Eclampsia ka matlab –
   (a) Hypoglycaemia (b) Pre-eclampsia + convulsions (c) Epilepsy (d) Only HTN
5. Eclampsia me drug of choice –
   (a) Magnesium sulphate (b) Phenytoin (c) Phenobarbitone (d) Diazepam
6. Pritchard regimen loading dose –
   (a) 1 g IM (b) 2 g IV (c) 4 g IV + 10 g IM (5 g each buttock) (d) 10 g IV
7. $MgSO_4$ toxicity ka antidote –
   (a) Atropine (b) Vitamin K (c) Naloxone (d) 10 % calcium gluconate 10 ml IV
8. $MgSO_4$ ki next dose se pehle check karte hain –
   (a) Knee jerk present, RR ≥ 16/min, urine ≥ 30 ml/hr (b) Only BP (c) Only FHS (d) Temperature
9. Toxicity me sabse pehle kya gayab hota hai?
   (a) Heart beat (b) Respiration (c) Patellar (knee) reflex (d) Consciousness
10. Pregnancy me antihypertensive first line –
    (a) Enalapril (b) Atenolol (c) Losartan (d) Labetalol
11. HELLP syndrome me "H" –
    (a) Hypertension (b) Haemorrhage (c) Haemolysis (d) Hyperglycaemia
12. GDM ka drug of choice –
    (a) Insulin (b) Sulphonylurea (c) Glibenclamide (d) Acarbose
13. Diabetic mother ke newborn me sabse common problem –
    (a) Hypercalcaemia (b) Hypoglycaemia (c) Hyperglycaemia (d) Anaemia
14. Rh isoimmunisation tab hota hai jab –
    (a) Mother Rh –ve, fetus Rh +ve (b) Mother Rh +ve, fetus Rh –ve (c) Dono Rh +ve (d) Dono Rh –ve
15. Anti-D dose delivery ke baad –
    (a) 50 µg IV (b) 300 µg IM within 72 hrs (c) 1000 µg (d) After 7 days
16. Twins me sabse common complication –
    (a) Preterm labour (b) Macrosomia (c) Post-term (d) Oligohydramnios
17. Hyperemesis gravidarum me dextrose se pehle kya dete hain (Wernicke rokne ke liye)?
    (a) Iron (b) Calcium (c) Thiamine (vitamin B1) (d) Vitamin C
18. IUD me X-ray sign (overlapping skull bones) –
    (a) Kehr's sign (b) Spalding's sign (c) Hegar's sign (d) Robert's sign

Level 2 – Priority & Application

19. Eclamptic fit ke dauran nurse ka pehla action –
(a) Mouth me spoon dalna (b) Restrain karna (c) Left lateral, airway clear, injury se bachav, help bulana (d) Oral fluids

20. $MgSO_4$ par patient ka RR 10/min aur knee jerk absent. Nurse –
(a) $MgSO_4$ rok do, calcium gluconate ready, doctor inform (b) Dose double (c) Ignore (d) Next dose de do

21. Severe pre-eclampsia ka sign –
(a) BP 130/80 (b) Weight gain 1 kg/ month (c) Mild ankle oedema (d) BP ≥ 160/110, headache, visual disturbance, epigastric pain, oliguria

22. Heart disease wali pregnant woman me sabse critical period –
(a) Delivery ke turant baad (b) 6 months postpartum (c) 1st trimester (d) 2nd trimester

23. Heart disease wali mother me 3rd stage me kaunsa drug avoid?
(a) Antibiotic (b) Oxytocin (c) Methylergometrine (d) Paracetamol

24. Heart disease mother ki labour me position –
(a) Prone (b) Propped up/ semi-Fowler's (c) Trendelenburg (d) Supine flat

25. Rh –ve mother ka ICT (indirect Coombs) –ve hai. 28 wks par –
(a) Antenatal anti-D prophylaxis (b) Kuch nahi (c) Termination (d) Exchange transfusion

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. $MgSO_4$ toxicity signs ka sahi order (pehle se baad) –
(a) Loss of patellar reflex → respiratory depression → cardiac arrest (b) Respiratory depression → reflex loss (c) Cardiac arrest → reflex loss (d) Sab ek saath

27. Match: P. Hydatidiform mole Q. Pre-eclampsia before 20 wks R. Polyhydramnios S. Macrosomia — with 1. Oesophageal atresia 2. Diabetes 3. Mole 4. Snowstorm
(a) P-4, Q-3, R-1, S-2 (b) P-3, Q-4, R-1, S-2 (c) P-4, Q-1, R-3, S-2 (d) P-2, Q-3, R-1, S-4

28. Assertion (A): ACE inhibitors pregnancy me contraindicated hain. Reason (R): Ye fetal renal failure aur oligohydramnios karte hain.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Pre-eclampsia ka risk factor NAHI hai –
(a) Twins (b) Second pregnancy, same partner, pichli pregnancy normal (c) Primigravida (d) Chronic HTN / diabetes

30. GDM screening (DIPSI) result 2 hr = 152 mg/dl. Interpretation –
(a) Overt diabetes (b) GDM (c) Hypoglycaemia (d) Normal

Answer Key & Explanation

1-d Hb < 11 g/dL (severe < 7).
2-d Iron deficiency.
3-c HTN (≥ 140/90) + proteinuria after 20 wks.
4-b Pre-eclampsia + convulsions.
5-a $MgSO_4$.
6-c 4 g IV (20 %) + 10 g IM (50 %).
7-d Calcium gluconate 10 ml of 10 % over 10 min.
8-a Teeno criteria.
9-c Patellar reflex 7–10 mEq/L par jata hai.
10-d Labetalol.
11-c Haemolysis, Elevated Liver enzymes, Low Platelets.
12-a Insulin – placenta cross nahi karta.
13-b Neonatal hypoglycaemia (fetal hyperinsulinaemia).
14-a Rh –ve mother, Rh +ve fetus.

15-b 300 µg within 72 hrs.
16-a Preterm labour.
17-c Thiamine – Wernicke's encephalopathy prevention.
18-b Spalding's sign.
19-c Left lateral, airway, safety.
20-a Toxicity – stop & antidote.
21-d Severe features.
22-a Immediately postpartum – autotransfusion.
23-c Methergine – sudden ↑ venous return → failure.
24-b Propped up.
25-a 28 wks antenatal anti-D (non-sensitised).
26-a Reflex loss → respiratory → cardiac.
27-a Mole – snowstorm; early pre-eclampsia – mole; polyhydramnios – oesophageal atresia; macrosomia – diabetes.
28-a Dono sahi, R reason hai.
29-b Same partner ke saath pichli normal pregnancy ke baad risk kam hota hai.
30-b ≥ 140 = GDM.

12 Bleeding in Pregnancy – Abortion, Ectopic, Mole, APH

Level 1 – Direct

1. Abortion ka sabse common cause (1st trimester) –
(a) Cervical incompetence (b) Infection (c) Trauma (d) Chromosomal abnormalities

2. Threatened abortion me cervical os –
(a) Dilated 4 cm (b) Open (c) Products bahar (d) Closed

3. Inevitable abortion me os –
(a) Normal (b) Closed (c) Absent (d) Open

4. Missed abortion –
(a) Heavy bleeding open os (b) Fetus dead, uterus me retained, os closed (c) Complete expulsion (d) Septic

5. Habitual (recurrent) abortion –
(a) 1 (b) ≥ 2 (c) ≥ 3 consecutive (d) ≥ 5

6. 2nd trimester recurrent painless abortion ka common cause –
(a) Thyroid (b) Fibroid (c) Chromosomal (d) Cervical incompetence

7. Cervical incompetence ka treatment –
(a) Cervical cerclage (McDonald/ Shirodkar) (b) Oxytocin (c) D&C (d) Hysterectomy

8. MTP Amendment Act 2021 – 1 RMP ki opinion se upper limit –
(a) 12 wks (b) 20 wks (c) 24 wks (d) 28 wks

9. Special categories (rape survivor, minor etc.) ke liye limit –
(a) No limit (b) 20 wks (c) 24 wks (2 RMPs) (d) 28 wks

10. Ectopic pregnancy ki sabse common site –
(a) Cervix (b) Abdomen (c) Ampulla of tube (d) Ovary

11. Ectopic ka sabse common risk factor –
(a) Multiparity (b) PID / tubal damage (c) Anaemia (d) Twins

12. Ruptured ectopic me shoulder tip pain –
(a) Cullen's sign (b) Kehr's sign (c) Hegar's sign (d) Chadwick's

13. Unruptured ectopic ka medical treatment –
(a) Insulin (b) Methotrexate (c) Misoprostol only (d) Oxytocin

14. H. mole USG appearance –
(a) Lemon sign (b) Ring of fire (c) Snowstorm / bunch of grapes (d) Double bubble

15. Complete mole ka karyotype –
(a) 45 XO (b) 46 XX (paternal) (c) 47 XXY (d) 69 XXY

16. Mole follow-up kis test se?
(a) AFP (b) Serum β-hCG (c) Urine sugar (d) Hb

17. APH –
(a) Menstruation (b) 28 wks (viability) ke baad delivery se pehle bleeding (c) 1st trimester bleeding (d) Delivery ke baad bleeding

18. Painless, causeless, recurrent bright red bleeding –
(a) Abruptio placentae (b) Vasa praevia (c) Rupture uterus (d) Placenta praevia

? Level 2 – Priority & Application

19. APH ke saath aayi mother me nurse kya NAHI karegi?
(a) IV line, blood grouping (b) Per vaginal examination (c) Pads count (d) Vitals & FHS

20. 32 wks par painful bleeding, uterus hard (woody), tender, FHS absent, BP high history. Diagnosis –
(a) Placenta praevia (b) Vasa praevia (c) Show (d) Abruptio placentae

21. Placenta praevia ka expectant (Macafee-Johnson) management kab possible?
(a) Fetal death (b) Heavy bleeding, 38 wks (c) Labour (d) Bleeding kam, mother stable, < 37 wks, fetus healthy

22. Ruptured ectopic, BP 80/50, pulse 130. First action –
(a) USG ka wait (b) Shock management – 2 wide-bore IV lines, blood, urgent laparotomy ki taiyari (c) Methotrexate (d) Oral fluids

23. Mole evacuation ke baad contraception advice –
(a) β-hCG follow-up ke dauran pregnancy avoid (OCP/ barrier), IUCD nahi (b) Turant pregnancy (c) Kabhi nahi (d) Sterilization

24. ARM ke turant baad painless bleeding aur acute fetal distress (sinusoidal FHR) – sochna chahiye –
(a) Cervicitis (b) Vasa praevia (c) Placenta praevia (d) Show

25. Septic abortion me nurse ki priority –
(a) Bed rest only (b) Discharge (c) Only analgesic (d) Antibiotics, IV fluids, vitals monitoring, evacuation ki taiyari

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Placenta praevia vs abruption – SAHI comparison –
(a) Praevia – painless, uterus soft, malpresentation common; Abruption – painful, tense uterus, PIH association (b) Dono painless (c) Abruption – soft uterus (d) Praevia – painful

27. Match: P. Threatened Q. Inevitable R. Incomplete S. Missed — with 1. Dead fetus retained, os closed 2. Os open, products partly passed 3. Os closed, fetus alive 4. Os open, no products passed
(a) P-1, Q-4, R-2, S-3 (b) P-4, Q-3, R-2, S-1 (c) P-3, Q-2, R-4, S-1 (d) P-3, Q-4, R-2, S-1

28. Assertion (A): Placenta praevia me PV exam contraindicated hai. Reason (R): PV exam se torrential haemorrhage ho sakti hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Abruptio placentae ki complication NAHI hai –
(a) Couvelaire uterus (b) DIC (c) Acute renal failure (d) Placenta accreta – sabse common

30. Ectopic ka classic triad NAHI hai –
(a) Amenorrhoea (b) Vaginal bleeding (c) High fever (d) Abdominal pain

✓ Answer Key & Explanation

1-d Chromosomal (~50 %).
2-d Os closed, fetus alive.
3-d Os open.
4-b Missed – dead retained, os closed.
5-c ≥ 3 consecutive (recurrent – ab ≥ 2 bhi likha jata hai).

6-d Cervical incompetence – painless 2nd trim.
7-a Cerclage 12–14 wks.
8-b 20 wks – 1 RMP.
9-c 20–24 wks – 2 RMPs.
10-c Ampulla (~70 %).
11-b PID / tubal damage.
12-b Kehr's – diaphragm irritation.
13-b Methotrexate.
14-c Snowstorm.
15-b 46 XX androgenetic.
16-b β-hCG.
17-b After 28 wks till delivery.
18-d Placenta praevia.
19-b PV exam nahi – praevia ho sakta.
20-d Abruption – painful, woody uterus.
21-d Expectant criteria.
22-b Shock + urgent surgery.
23-a hCG normal hone tak pregnancy avoid.
24-b Vasa praevia – fetal blood loss.
25-d Sepsis management.
26-a Sahi comparison.
27-d Sahi.
28-a Dono sahi, R reason hai.
29-d Accreta abruption ki complication nahi.
30-c Triad – amenorrhoea, pain, bleeding.

13 Abnormal Labour & Malpresentations

Level 1 – Direct

1. Sabse common malpresentation –
   (a) Breech (b) Shoulder (c) Face (d) Brow
2. Breech ka sabse common type –
   (a) Complete (flexed) (b) Knee (c) Footling (d) Frank (extended legs)
3. ECV (External cephalic version) kab karte hain?
   (a) 28 wks (b) In labour (c) 36–37 wks (d) 32 wks
4. Breech me after-coming head ke liye manoeuvre –
   (a) Woods screw (b) McRoberts (c) Ritgen's (d) Mauriceau-Smellie-Veit
5. Breech me extended arms ke liye –
   (a) Lovset's manoeuvre (b) Zavanelli (c) Burns-Marshall (d) MSV
6. Face presentation ka denominator –
   (a) Occiput (b) Sinciput (c) Sacrum (d) Mentum (chin)
7. Brow presentation me management (persistent, term) –
   (a) Forceps (b) Vaginal delivery (c) LSCS (d) Vacuum
8. Transverse lie me presenting part –
   (a) Breech (b) Shoulder (c) Vertex (d) Face
9. OP position me mother ki common complaint –
   (a) Headache (b) Severe backache (c) Itching (d) Fever
10. Obstructed labour ka sign – pathological retraction ring –
    (a) Schroeder's ring (b) Constriction ring (c) Bandl's ring (d) Contraction ring normal
11. Prolonged labour –
    (a) > 6 hrs (b) > 12 hrs (c) > 18–24 hrs (d) > 48 hrs
12. Precipitate labour –
    (a) < 1 hr (b) < 3 hrs (c) < 6 hrs (d) < 12 hrs
13. Preterm labour –
    (a) Before 40 wks (b) Before 28 wks (c) After 42 wks (d) Before 37 completed wks
14. Post-term pregnancy –
    (a) ≥ 40 wks (b) ≥ 41 wks (c) ≥ 42 wks (d) ≥ 44 wks
15. PROM –
    (a) Placenta ke saath (b) 2nd stage me rupture (c) Labour me ARM (d) Labour shuru hone se pehle membranes ka rupture
16. Preterm labour me first-line tocolytic –
    (a) Oxytocin (b) Nifedipine (c) Misoprostol (d) Methergine
17. Fetal lung maturity ke liye steroid –
    (a) Insulin (b) Prednisolone oral (c) Dexamethasone 6 mg IM 12-hrly × 4 doses (d) Hydrocortisone
18. Bishop score favourable –
    (a) 0 (b) 2 (c) ≤ 4 (d) ≥ 8

Level 2 – Priority & Application

19. PROM wali mother me nurse kya avoid karegi?
(a) Temperature monitoring (b) FHS (c) Pad check (d) Repeated PV exams

20. Obstructed labour me kya contraindicated hai?
(a) Antibiotics (b) Oxytocin (c) Catheterisation (d) IV fluids

21. Obstructed labour ke baad VVF rokne ke liye –
(a) Continuous bladder drainage (catheter) 10–14 days (b) Fluid restriction (c) Pessary (d) Early discharge

22. Oxytocin infusion par contractions 1 min se zyada lambe aur 2 min me 5 se zyada. FHS 100. Nurse –
(a) Ignore (b) Drip badhana (c) Walk karana (d) Oxytocin turant band, left lateral, O_2, doctor inform

23. Breech delivery me body deliver hone ke baad baby ko –
(a) Hands-off – apne aap umbilicus tak aane dena, back anterior rakhna (b) Rotate 360° (c) Pull karna (d) Fundal pressure hamesha

24. Preterm labour (32 wks) me admission par nurse ki priority –
(a) Ambulation (b) Steroid & tocolysis order follow, $MgSO_4$ neuroprotection, NICU inform (c) Discharge (d) Enema

25. Membranes ruptured, cord vagina me dikh raha – ye kis ke saath zyada hota hai?
(a) Post-term (b) Malpresentation / high presenting part / polyhydramnios (c) OP only (d) Engaged vertex

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Bishop score ke components NAHI hai –
(a) Fetal heart rate (b) Station (c) Dilatation (d) Effacement

27. Match: P. Vertex Q. Face R. Breech S. Shoulder — with 1. Acromion 2. Sacrum 3. Mentum 4. Occiput
(a) P-1, Q-3, R-2, S-4 (b) P-4, Q-2, R-3, S-1 (c) P-4, Q-3, R-2, S-1 (d) P-3, Q-4, R-2, S-1

28. Assertion (A): Mento-posterior face presentation me vaginal delivery possible nahi. Reason (R): Head already maximally extended hai, aur extend nahi ho sakta.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Bishop: dilatation 3 cm (2), effacement 60 % (2), station −1 (2), soft (2), anterior (2). Score –
(a) 6 (b) 8 (c) 10 (d) 12

30. Induction of labour ka contraindication NAHI hai –
(a) Post-term pregnancy (b) Previous classical CS (c) Transverse lie (d) Placenta praevia

Answer Key & Explanation

1-a Breech (~3 % at term).
2-d Frank breech.
3-c 36–37 wks.
4-d MSV (jaw flexion shoulder traction).
5-a Lovset's.
6-d Mentum.
7-c Mento-vertical 14 cm – LSCS.
8-b Shoulder (acromion).
9-b Backache – occiput sacrum par.
10-c Bandl's ring.
11-c > 18–24 hrs.
12-b < 3 hrs.
13-d < 37 completed wks.
14-c ≥ 42 wks.
15-d Before onset of labour.
16-b Nifedipine.
17-c Dexamethasone 6 mg × 4.
18-d ≥ 8 favourable (≤ 6 unfavourable).

19-d PV – infection risk.
20-b Oxytocin – rupture uterus.
21-a Continuous catheter.
22-d Hyperstimulation – stop oxytocin.
23-a "Hands off the breech".
24-b Steroids, tocolysis, neuroprotection.
25-b Cord prolapse risk factors.
26-a FHR Bishop me nahi (consistency, position hote hain).
27-c Sahi denominators.
28-a Dono sahi, R reason hai.
29-c 2+2+2+2+2 = 10.
30-a Post-term – induction ka indication hai.

14 Obstetric Emergencies & PPH

Level 1 – Direct

1. PPH (vaginal delivery) –
   (a) ≥ 200 ml (b) ≥ 500 ml (c) ≥ 1000 ml (d) ≥ 1500 ml
2. CS ke baad PPH –
   (a) ≥ 300 ml (b) ≥ 500 ml (c) ≥ 1000 ml (d) ≥ 2000 ml
3. PPH ka sabse common cause –
   (a) Retained tissue (b) Thrombin (coagulopathy) (c) Trauma (d) Uterine atony
4. Primary PPH ka time –
   (a) 6 wks ke baad (b) 24 hrs se 6 wks (c) Delivery ke 24 hrs ke andar (d) Antenatal
5. Secondary PPH ka sabse common cause –
   (a) Atony (b) Retained bits of placenta/ membranes + infection (c) Tear (d) DIC
6. 4 T's me "Tone" kis ko darshata hai?
   (a) Thrombin (b) Atony (c) Tears (d) Tissue
7. Retained placenta – placenta deliver nahi hua –
   (a) 2 hrs (b) 6 hrs (c) 10 min (d) 30 min
8. Carboprost kis me contraindicated?
   (a) Hypertension (b) Bronchial asthma (c) Anaemia (d) Diabetes
9. Methylergometrine kis me contraindicated?
   (a) Asthma (b) Hypertension / heart disease (c) Diabetes (d) Epilepsy
10. Tranexamic acid PPH me –
    (a) 1 g IV within 3 hrs of birth (b) After 24 hrs (c) 10 g oral (d) IM only
11. Uterine inversion me sign – "shock out of proportion to blood loss" kis karan?
    (a) Septic (b) Anaphylactic (c) Cardiogenic (d) Neurogenic (vagal) – ligament traction
12. Uterine inversion replace karne ka manoeuvre –
    (a) Ritgen (b) Lovset (c) McRoberts (d) Johnson's
13. Rupture uterus ka sabse common cause (aaj) –
    (a) Twins (b) Forceps (c) Trauma (d) Previous CS scar
14. Cord prolapse me mother ki position –
    (a) Fowler's (b) Knee-chest / exaggerated Sims (c) Lithotomy (d) Supine
15. Shoulder dystocia ka sign –
    (a) Chadwick (b) Spalding (c) Kehr (d) Turtle sign
16. Shoulder dystocia me first manoeuvre –
    (a) Cleidotomy (b) Zavanelli (c) McRoberts + suprapubic pressure (d) Fundal pressure
17. DIC ka sabse common obstetric cause –
    (a) Placenta praevia (b) Abruptio placentae (c) Breech (d) Twins
18. Amniotic fluid embolism ki presentation –
    (a) Sudden dyspnoea, cyanosis, hypotension, collapse, DIC (b) Oedema (c) Slow fever (d) Itching

Level 2 – Priority & Application

19. Delivery ke baad heavy bleeding, uterus soft. Nurse ka FIRST step –
(a) Mother ko sitting (b) Help bulana + uterine (fundal) massage (c) Blood test lena (d) Doctor ka wait

20. Uterus firm hai par bright red bleeding continue. Sabse likely cause –
(a) Normal lochia (b) Retained placenta (c) Genital tract tear (trauma) (d) Atony

21. Cord prolapse diagnose hua. Nurse ka immediate action –
(a) Hand se presenting part ko upar push karke cord se pressure hatana, help bulana, urgent CS ki taiyari (b) Cord ko haath me dabana (c) Cord ko andar push karna (d) Mother ko walk

22. Prolapsed cord bahar ho to –
(a) Warm moist saline pad se cover karo, handling minimum (b) Cut karo (c) Dry chhod do (d) Ice lagao

23. PPH me bimanual uterine compression kab?
(a) Sirf CS me (b) Kabhi nahi (c) Massage aur uterotonics ke baad bhi bleeding continue (d) Pehla step

24. PPH me Non-pneumatic anti-shock garment (NASG) ka role –
(a) Lactation (b) Referral/ transport tak shock control (c) Pain (d) Infection rokna

25. PPH se shock me mother ki position –
(a) Fowler's (b) Legs raised (head low/ leg elevation), warm rakhna (c) Prone (d) Sitting

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Match: P. Tone Q. Trauma R. Tissue S. Thrombin — with 1. Coagulopathy 2. Retained placenta 3. Tears/ rupture 4. Atony
(a) P-1, Q-3, R-2, S-4 (b) P-4, Q-3, R-2, S-1 (c) P-4, Q-2, R-3, S-1 (d) P-3, Q-4, R-2, S-1

27. HELPERR (shoulder dystocia) me "E" –
(a) Elevate legs (b) Emergency CS (c) Evaluate for episiotomy (d) Expel

28. Assertion (A): PPH me pehle bladder empty kiya jata hai. Reason (R): Full bladder uterus ke contraction me rukawat dalta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Rupture uterus ke signs me NAHI hai –
(a) Loss of station (b) Fetal parts easily palpable (c) Fetal distress (earliest) (d) Contractions zyada strong ho jate hain

30. Shoulder dystocia me kya NAHI karna chahiye?
(a) Delivery of posterior arm (b) Suprapubic pressure (c) McRoberts (d) Fundal pressure

Answer Key & Explanation

1-b ≥ 500 ml vaginal.
2-c ≥ 1000 ml CS.
3-d Atony (~70–80 %).
4-c 24 hrs ke andar.
5-b Retained products + infection.
6-b Tone = atony.
7-d > 30 min.
8-b Asthma – bronchospasm.
9-b HTN/ heart disease.
10-a 1 g IV within 3 hrs (WOMAN trial).
11-d Neurogenic shock.
12-d Johnson's manoeuvre.
13-d Scar rupture.
14-b Knee-chest / Sims – cord pressure kam.
15-d Turtle sign.
16-c McRoberts + suprapubic pressure.
17-b Abruption.
18-a Sudden collapse.
19-b Call for help + massage.
20-c Firm uterus + bleeding = trauma.

21-a Pressure relieve, urgent delivery.
22-a Warm moist pad – vasospasm rokna.
23-c Refractory atony.
24-b NASG – transport tak.
25-b Leg elevation, warmth.
26-b Sahi 4 T's.
27-c Evaluate episiotomy.
28-a Dono sahi, R reason hai.
29-d Rupture ke baad contractions ruk jate hain.
30-d Fundal pressure impaction badhata hai.

15 Puerperal Complications & Neonatal Problems

? Level 1 – Direct

1. Puerperal sepsis ka sabse common organism (classical, severe) –
(a) Candida (b) Virus (c) Group A β-haemolytic streptococcus (d) Trichomonas

2. Puerperal sepsis ka sabse common site –
(a) Breast (b) Endometritis (placental site) (c) Lungs (d) Brain

3. "6 cleans" kis ke liye?
(a) Anaemia (b) Contraception (c) Puerperal & neonatal sepsis prevention (d) Vaccination

4. Subinvolution ka sign –
(a) Uterus bada, soft, lochia zyada/ lambe samay (b) Uterus chhota (c) Fever only (d) Lochia absent

5. Postpartum blues ki duration –
(a) 2 wks se kam (self-limiting) (b) Lifelong (c) 6 months (d) 1 year

6. Puerperal psychosis –
(a) Self-limiting in 2 days (b) Only physical (c) Normal (d) Psychiatric emergency

7. Sheehan's syndrome ka pehla sign –
(a) Fever (b) Lactation failure (c) Weight gain (d) Hypertension

8. RDS (hyaline membrane disease) ka cause –
(a) Infection (b) Surfactant deficiency (c) Meconium (d) Heart disease

9. RDS sabse zyada kis me?
(a) Post-term (b) Preterm baby (c) Term normal (d) Large baby

10. Meconium aspiration syndrome kis me common hai?
(a) Twins (b) Post-term / fetal distress (c) Preterm 28 wks (d) Breech only

11. Neonatal sepsis ka earliest sign –
(a) Diarrhoea (b) High fever (c) Rash (d) Refusal to feed / lethargy

12. Erb's palsy me nerve roots –
(a) S2–S4 (b) C8–T1 (c) C5–C6 (d) L4–L5

13. Klumpke's palsy –
(a) C8–T1 – claw hand (b) Phrenic (c) Facial nerve (d) C5–C6

14. Erb's palsy me hand ki position –
(a) Normal (b) Wrist drop (c) Waiter's tip (d) Claw hand

15. Sabse common birth fracture –
(a) Skull (b) Clavicle (c) Femur (d) Humerus

16. Neural tube defect rokta hai –
(a) Iron (b) Vitamin D (c) Folic acid (d) Calcium

17. Hypospadias me urethral opening –
(a) Penis ke ventral (neeche) side (b) Normal (c) Dorsal side (d) Absent

18. Cleft lip repair ka usual age –
(a) 3 months (rule of 10s approx) (b) 2 years (c) Birth par (d) 5 years

? Level 2 – Priority & Application

19. Postnatal mother day 3 par 39 °C fever, foul lochia, tender uterus. Nurse ka first action –
(a) Discharge (b) Ice pack (c) Vitals, doctor inform, culture, antibiotics shuru karne ki taiyari, Fowler's position (d) Feeding stop

20. Postnatal mother ko calf pain, swelling, Homan's sign +ve. Nurse kya NAHI karegi?
(a) Doctor inform (b) Anticoagulant order follow (c) Bed rest with leg elevation (d) Leg massage

21. Puerperal sepsis me position –
(a) Trendelenburg (b) Fowler's / semi-sitting – lochia drainage (c) Supine flat (d) Prone

22. Mother kehti hai "mujhe awaze sunai deti hain, baby ko nuksaan pahunchane ka mann karta hai". Nurse –
(a) Discharge (b) Counselling next week (c) Mother aur baby ki safety, akela na chhodna, turant psychiatrist referral (d) Ignore

23. Neonatal hypoglycaemia (< 45 mg/dl) – symptomatic baby me –
(a) 25 % dextrose (b) 10 % dextrose 2 ml/kg IV (c) Wait (d) Insulin

24. Spina bifida (meningomyelocele) newborn ki pre-op care –
(a) Supine position (b) Dry dressing tight (c) Sac par sterile moist saline dressing, prone position (d) Sac open chhodna

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

25. Match: P. Erb's palsy Q. Klumpke's palsy R. Facial palsy S. Phrenic nerve palsy — with 1. Forceps 2. C5–C6 3. Respiratory distress 4. C8–T1
(a) P-4, Q-2, R-1, S-3 (b) P-3, Q-4, R-1, S-2 (c) P-2, Q-4, R-1, S-3 (d) P-2, Q-1, R-4, S-3

26. Assertion (A): Antenatal corticosteroids preterm babies me RDS ko kam karte hain. Reason (R): Steroids fetal lungs me surfactant production badhate hain.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

27. Postpartum depression ke liye kaun SAHI NAHI hai?
(a) Self-harm risk (b) Usually 2 wks ke baad continue (c) Ye sirf 1 din ki normal problem hai (d) EPDS se screen

28. Tracheo-oesophageal fistula ka newborn sign –
(a) Excessive frothing/ drooling, feeding par choking & cyanosis (b) Jaundice (c) Bilious vomiting (d) Projectile vomiting 3 wks

29. Puerperal sepsis ke risk factor me NAHI aata –
(a) Prolonged labour (b) Repeated PV (c) PROM (d) Exclusive breastfeeding

30. Hydrocephalus ka sign NAHI hai –
(a) Head circumference ↑ (b) Sunset eyes (c) Sunken fontanelle (d) Bulging fontanelle

✓ Answer Key & Explanation

1-c GAS – severe sepsis (common mixed – anaerobes, E. coli).

2-b Endometritis.

3-c Clean hands, surface, blade, cord tie, cloth,

perineum.
4-a Bulky soft uterus, persistent lochia.
5-a < 2 wks.
6-d Emergency – infanticide/ suicide risk.
7-b Lactation failure (prolactin ↓).
8-b Surfactant deficiency.
9-b Preterm.
10-b Post-term/ distress.
11-d Refusal to feed.
12-c C5–C6.
13-a C8–T1.
14-c Waiter's tip.
15-b Clavicle.
16-c Folic acid.
17-a Ventral.
18-a ~3 months (palate 9–18 months).
19-c Sepsis workup & treatment.
20-d DVT me massage nahi – embolism.
21-b Fowler's.
22-c Puerperal psychosis – safety, referral.
23-b 10 % D 2 ml/kg.
24-c Prone, moist sterile dressing.
25-c Sahi.
26-a Dono sahi, R reason hai.
27-c PPD serious hai – 1 din ki baat nahi.
28-a TOF – frothing, choking.
29-d Breastfeeding risk factor nahi.
30-c Sunken – dehydration.

16 Obstetric Operations & Labour Analgesia

Level 1 – Direct

1. Sabse common CS type –
   (a) T-shaped (b) Extraperitoneal (c) Classical (d) LSCS (transverse lower segment)
2. Aaj ke samay me CS ka sabse common indication –
   (a) Eclampsia (b) Twins (c) Previous CS (d) Breech
3. VBAC kis CS ke baad contraindicated hai?
   (a) Classical CS (b) LSCS (c) Kisi ke baad nahi (d) Hamesha allowed
4. Mendelson's syndrome –
   (a) Uterine rupture (b) DVT (c) Spinal headache (d) Acid aspiration pneumonitis under GA
5. CS ke pehle prophylactic antibiotic kab?
   (a) 24 hrs baad (b) Never (c) Post-op day 3 (d) Incision se 60 min pehle
6. CS me Foley catheter kyon?
   (a) Infection ke liye (b) Sedation (c) Bladder injury se bachav (d) Comfort
7. Forceps ka inventor –
   (a) Kielland (b) Malmström (c) Smellie (d) Chamberlen
8. Vacuum (ventouse) ka inventor (metal cup) –
   (a) Chamberlen (b) Malmström (c) Simpson (d) Wrigley
9. Rotational forceps –
   (a) Wrigley's (b) Piper's (c) Simpson's (d) Kielland's
10. After-coming head of breech ke liye forceps –
    (a) Kielland's (b) Wrigley's (c) Piper's (d) Barnes
11. Vacuum se baby me artificial caput –
    (a) Cephalhaematoma (b) Moulding (c) Chignon (d) Caput
12. MVA kitne hafte tak?
    (a) 6 wks (b) 12 wks (c) 20 wks (d) 24 wks
13. D&C ki late complication (intrauterine adhesions) –
    (a) Sheehan's (b) Meigs' (c) Turner's (d) Asherman's syndrome
14. Cervical cerclage kab remove karte hain?
    (a) After delivery (b) 34 wks (c) 28 wks (d) 37 wks / labour onset
15. Labour me sabse effective analgesia –
    (a) TENS (b) Epidural (c) Entonox (d) Pethidine
16. Epidural ka sabse common side effect –
    (a) Hypertension (b) Fever only (c) Hypotension (d) Seizures
17. Pudendal block me nerve roots –
    (a) T10–T12 (b) L1–L2 (c) S2–S4 (d) C5–C6
18. Entonox me gases –
    (a) $CO_2 + O_2$ (b) 100 % O_2 (c) 21 % O_2 (d) 50 % N_2O + 50 % O_2

Level 2 – Priority & Application

19. CS ke baad pehle 24 hrs me nurse ki priority –
(a) Vitals, uterine tone & vaginal bleeding, urine output, dressing (b) Discharge teaching only (c) Diet chart (d) Bath

20. Pethidine delivery ke kitne paas avoid?
(a) 30 min (b) Never avoid (c) 2–4 hrs ke andar (neonatal respiratory depression) (d) 24 hrs

21. Pethidine se baby ki respiratory depression ka antidote –
(a) Atropine (b) Naloxone (c) Calcium (d) Adrenaline

22. Forceps delivery se pehle nurse kya ensure kare?
(a) Oxytocin stop (b) Enema (c) Bladder empty, full dilatation, membranes ruptured, consent (d) Mother ko bhookha

23. Post-CS mother ko breastfeeding ke liye convenient position –
(a) Standing (b) Prone (c) Football hold / side-lying (d) Cross-legged

24. Epidural ke baad BP 85/50. First nurse action –
(a) Epidural badhana (b) Sitting (c) Left lateral tilt, IV fluids fast, O_2, anaesthetist inform (d) Ignore

25. ECV ke baad Rh –ve mother ko –
(a) Nothing (b) Methergine (c) Oxytocin (d) Anti-D

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Vacuum ka contraindication NAHI hai –
(a) Face presentation (b) Preterm < 34 wks (c) Fully dilated cervix, vertex, engaged head (d) Breech

27. Match: P. Wrigley's Q. Kielland's R. Piper's S. Simpson's — with 1. After-coming head 2. Outlet forceps 3. Rotational 4. Mid/ low cavity
(a) P-3, Q-2, R-1, S-4 (b) P-4, Q-3, R-1, S-2 (c) P-2, Q-3, R-1, S-4 (d) P-2, Q-1, R-3, S-4

28. Assertion (A): Classical CS ke baad agle pregnancy me VBAC nahi karte. Reason (R): Upper segment scar me labour ke dauran (aur pehle bhi) rupture ka risk zyada hota hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Classical CS ka indication NAHI hai –
(a) Previous one LSCS (b) Perimortem CS (c) Lower segment par large fibroid (d) Transverse lie with back down

30. Lignocaine (without adrenaline) max dose 3 mg/kg. 60 kg mother me max approx –
(a) 60 mg (b) 180 mg (c) 300 mg (d) 600 mg

Answer Key & Explanation

1-d LSCS ~99 %.
2-c Previous CS.
3-a Classical CS.
4-d Acid aspiration.
5-d Within 60 min before incision.
6-c Bladder injury prevention.
7-d Chamberlen family.
8-b Malmström (1954).
9-d Kielland's.
10-c Piper's.
11-c Chignon.
12-b 12 wks.
13-d Asherman's.
14-d 37 wks.
15-b Epidural.
16-c Hypotension.
17-c S2–S4.
18-d 50/50.

19-a PPH aur post-op complications.
20-c 2–4 hrs.
21-b Naloxone.
22-c Prerequisites.
23-c Wound par pressure nahi.
24-c Hypotension management.
25-d Anti-D – fetomaternal haemorrhage risk.
26-c Ye to prerequisites hain.
27-c Sahi.
28-a Dono sahi, R reason hai.
29-a Previous LSCS – repeat LSCS/ VBAC.
30-b 3 × 60 = 180 mg.

17 Drugs in Obstetrics

Level 1 – Direct

1. PPH prevention & treatment ka drug of choice –
   (a) Oxytocin (b) Carboprost (c) Methergine (d) Misoprostol
2. Oxytocin ki high dose ka side effect (ADH-like) –
   (a) Water intoxication (b) Hypothermia (c) Hyperglycaemia (d) Bronchospasm
3. Oxytocin ka storage –
   (a) Sunlight (b) 2–8 °C refrigerator (c) Freezer (d) Kahin bhi
4. Methergine ki dose –
   (a) 0.2 mg IM (b) 2 mg IM (c) 10 IU (d) 20 mg
5. Carboprost (15-methyl PGF2α) dose –
   (a) 2.5 mg (b) 25 µg (c) 250 µg deep IM (d) 25 mg
6. Carboprost ka sabse common side effect –
   (a) Diarrhoea (b) Constipation (c) Bradycardia (d) Hypertension
7. Community level PPH ke liye heat-stable uterotonic –
   (a) Misoprostol (b) Carboprost (c) Methergine (d) Oxytocin
8. PPH treatment me misoprostol dose –
   (a) 25 µg (b) 100 µg (c) 800 µg sublingual (d) 2000 µg
9. Cervical ripening gel (PGE2) –
   (a) Misoprostol (b) Carboprost (c) Mifepristone (d) Dinoprostone (Cerviprime)
10. Mifepristone kya hai?
    (a) Oxytocic (b) Antiprogesterone (c) Tocolytic (d) Prostaglandin
11. Indomethacin tocolysis ka fetal side effect –
    (a) Polyhydramnios (b) Premature closure of ductus arteriosus (c) Jaundice (d) Macrosomia
12. β-agonist tocolytic ka side effect –
    (a) Bradycardia (b) Hypoglycaemia (c) Constipation (d) Maternal tachycardia, pulmonary oedema, hyperglycaemia
13. Pregnancy me safe anticoagulant –
    (a) Warfarin (b) Heparin / LMWH (c) Aspirin high dose (d) Dabigatran
14. Pre-eclampsia prevention (high risk) –
    (a) Heparin (b) Diuretics (c) Steroid (d) Low-dose aspirin 75–150 mg from 12–16 wks
15. Gray baby syndrome –
    (a) Tetracycline (b) Chloramphenicol (c) Penicillin (d) Streptomycin
16. Teeth staining & bone growth defect –
    (a) Penicillin (b) Tetracycline (c) Cephalexin (d) Erythromycin
17. Streptomycin pregnancy me –
    (a) Kernicterus (b) Gray baby (c) Fetal ototoxicity (8th nerve) (d) Safe
18. Epilepsy wali pregnant woman me kaunsa antiepileptic avoid (NTD)?
    (a) None (b) Lamotrigine (c) Levetiracetam (d) Valproate

Level 2 – Priority & Application

19. Oxytocin infusion par mother ko headache, vomiting, confusion. Nurse sochegi –
(a) Eclampsia only (b) Water intoxication – infusion rokna, intake-output, doctor inform (c) Normal labour (d) Hypoglycaemia

20. PPH me asthma patient ko kaunsa drug nahi dena chahiye?
(a) Misoprostol (b) Tranexamic acid (c) Carboprost (d) Oxytocin

21. Pre-eclampsia mother ki PPH me kaunsa avoid?
(a) Oxytocin (b) Carboprost (if no asthma) (c) Misoprostol (d) Methergine

22. Dinoprostone gel lagane ke baad mother ko –
(a) Turant walk (b) 30 min lete rehna, CTG/ contractions monitor (c) Bath (d) Discharge

23. Breastfeeding mother me kaunsa drug contraindicated hai?
(a) Insulin (b) Penicillin (c) Radioactive iodine / cytotoxic drugs (d) Paracetamol

24. Anti-D ka route –
(a) Oral (b) IV push (c) Deep IM (deltoid) (d) SC

25. Labetalol kis me caution/ avoid?
(a) Bronchial asthma (b) Twins (c) Anaemia (d) Diabetes

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Match: P. Oxytocin Q. Methergine R. Carboprost S. Misoprostol — with 1. PGE1 2. Ergot alkaloid 3. Posterior pituitary hormone 4. PGF2α
(a) P-3, Q-2, R-4, S-1 (b) P-1, Q-2, R-4, S-3 (c) P-3, Q-4, R-2, S-1 (d) P-2, Q-3, R-4, S-1

27. Carboprost max total dose (8 doses × 250 µg) –
(a) 1 mg (b) 2 mg (c) 4 mg (d) 8 mg

28. Assertion (A): Methergine baby ki delivery se pehle nahi dena chahiye. Reason (R): Ye sustained tonic uterine contraction karta hai jisse fetus trap/ hypoxic ho sakta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Oxytocin ka contraindication NAHI hai –
(a) CPD (b) Transverse lie (c) Previous classical CS (d) Uterine inertia (hypotonic contractions) with no obstruction

30. Assertion (A): Insulin GDM me drug of choice hai. Reason (R): Insulin placenta cross nahi karta.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

Answer Key & Explanation

1-a Oxytocin.
2-a Water intoxication.
3-b 2–8 °C (heat-stable carbetocin alag).
4-a 0.2 mg IM.
5-c 250 µg deep IM.
6-a Diarrhoea.
7-a Misoprostol.
8-c 800 µg SL.
9-d Dinoprostone.
10-b Antiprogesterone (RU-486).
11-b Ductus closure, oligohydramnios.
12-d β-agonist side effects.
13-b Heparin – placenta cross nahi.
14-d Low-dose aspirin.
15-b Chloramphenicol.
16-b Tetracycline.
17-c Ototoxicity.
18-d Valproate.

19-b Water intoxication.
20-c Carboprost – bronchospasm.
21-d Methergine – BP ↑.
22-b 30 min lete rahe.
23-c Radioactive iodine/ cytotoxics.
24-c Deep IM.
25-a Non-selective β-blocker – asthma.
26-a Sahi.
27-b 8 × 250 µg = 2 mg.
28-a Dono sahi, R reason hai.
29-d Hypotonic inertia – augmentation ka indication.
30-a Dono sahi, R reason hai.

18 Family Planning & Contraception

Level 1 – Direct

1. Contraceptive failure measure –
(a) Bishop score (b) APGAR (c) Pearl index (failures per 100 woman-years) (d) Body mass index

2. Cu-T 380A ki effective life –
(a) 1 yr (b) 3 yrs (c) 5 yrs (d) 10 yrs

3. Cu 375 ki life –
(a) 3 yrs (b) 5 yrs (c) 10 yrs (d) 12 yrs

4. Interval IUCD insertion ka best time –
(a) Pregnancy me (b) Menstruation ke dauran / within 12 days of LMP (c) Mid-cycle only (d) Kabhi bhi bina check

5. PPIUCD placenta delivery ke kitne time ke andar?
(a) 1 week (b) 6 wks (c) 10 min (d) 48 hrs

6. IUCD ka sabse common side effect –
(a) Infection (b) Ectopic (c) Bleeding & pain (d) Perforation

7. Mala-N pack me –
(a) 21 iron (b) 28 hormonal (c) 7 hormonal (d) 21 hormonal + 7 iron tablets

8. Chhaya (Centchroman/ Ormeloxifene) –
(a) Non-steroidal, weekly pill (after 3 months twice weekly) (b) Implant (c) Daily steroidal pill (d) Injection

9. Antara injectable –
(a) Yearly (b) Weekly (c) DMPA 150 mg IM every 3 months (d) Monthly

10. Emergency contraceptive pill (LNG) –
(a) After 7 days (b) 1.5 mg within 72 hrs (c) 10 mg (d) 0.15 mg daily

11. Emergency contraception ke liye sabse effective method (5 days tak) –
(a) Calendar (b) Withdrawal (c) Cu-IUCD (d) Condom

12. LAM ke criteria –
(a) Menses resumed (b) Baby 1 yr (c) Partial feeding (d) Exclusive breastfeeding + amenorrhoea + baby < 6 months

13. Male condom ka brand (free supply) –
(a) Mala-D (b) Nirodh (c) Saheli (d) Antara

14. Tubectomy ki sabse common technique –
(a) Irving (b) Uchida (c) Pomeroy (d) Madlener

15. Vasectomy ke baad kitne time condom use?
(a) 1 week (b) No need (c) 1 year (d) 3 months / negative semen analysis tak

16. No-scalpel vasectomy –
(a) Female method (b) Male sterilization method (c) IUCD (d) Pill

17. Combined OCP ka mechanism –
(a) Barrier (b) Ovulation inhibition (c) Implantation only (d) Sperm kill

18. Natural method (calendar) me safe period ki calculation (Ogino-Knaus) –
(a) Shortest cycle − 18 = first fertile day; longest − 11 = last fertile day (b) Day 14 only (c) Longest − 18 (d) Shortest − 11

? Level 2 – Priority & Application

19. OCP leti woman ko severe chest pain, calf pain, severe headache, blurred vision ("ACHES"). Nurse –
(a) Painkiller (b) OCP continue (c) OCP band karke turant doctor/ hospital (d) Ignore

20. Combined OCP kis me contraindicated?
(a) Acne (b) Anaemia (c) Dysmenorrhoea (d) Breast cancer, DVT history, smoker > 35 yrs, liver disease

21. Breastfeeding mother (6 wk) ke liye best hormonal pill –
(a) High-dose oestrogen (b) None (c) Progestin-only pill (POP) (d) Combined OCP

22. IUCD user ko kis sign par turant aana chahiye?
(a) Mild cramps 1 din (b) Missed period, severe abdominal pain, foul discharge, fever, thread na milna (c) Kuch nahi (d) Halka spotting first month

23. Woman 1 OCP pill bhool gayi (12 hrs late). Advice –
(a) Yaad aate hi le lo, agli pill normal time par (b) 4 pills ek saath (c) Pack chhod do (d) Ab use band

24. Unprotected intercourse 4 din pehle hua. Kya option hai?
(a) OCP pack (b) LNG 1.5 mg (only 72 hrs) (c) Kuch nahi (d) Cu-IUCD within 5 days

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

25. Match: P. Mala-N Q. Chhaya R. Antara S. Cu-T 380A — with 1. DMPA 3-monthly 2. 10 yrs IUCD 3. Weekly non-steroidal 4. Combined OCP
(a) P-3, Q-4, R-1, S-2 (b) P-4, Q-3, R-1, S-2 (c) P-4, Q-1, R-3, S-2 (d) P-2, Q-3, R-1, S-4

26. IUCD ka contraindication NAHI hai –
(a) Unexplained vaginal bleeding (b) Multiparous woman wanting spacing (c) Suspected pregnancy (d) Active PID

27. Cycles 26 se 32 din. Calendar method se fertile period –
(a) Day 8 to day 21 (b) Day 14 only (c) Day 10–18 (d) Day 1–7

28. Assertion (A): LAM 6 months ke baad reliable nahi rehta. Reason (R): Baby ko complementary feeding shuru hone se suckling kam hoti hai aur ovulation return ho sakta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Spacing method NAHI hai –
(a) Condom (b) IUCD (c) Tubectomy (d) OCP

30. Pearl index = (failures × 1200) / total months of exposure. 100 women 1 yr me 3 pregnancies –
(a) 0.3 (b) 3 (c) 12 (d) 30

✓ Answer Key & Explanation

1-c Pearl index.
2-d 10 yrs.
3-b 5 yrs.
4-b Menses/ within 12 days.
5-c Within 10 min (post-placental) – 48 hrs tak PPIUCD.
6-c Bleeding & pain – removal ka common reason.
7-d 21 + 7 ferrous fumarate.
8-a Twice weekly × 3 months, phir weekly.

9-c DMPA 150 mg 3-monthly.
10-b 1.5 mg within 72 hrs.
11-c Cu-IUCD within 5 days.
12-d LAM criteria.
13-b Nirodh.
14-c Pomeroy.
15-d 3 months/ azoospermia.
16-b NSV – male.
17-b Ovulation inhibition.
18-a Shortest − 18, longest − 11.
19-c ACHES – serious – stop & refer.
20-d Absolute contraindications.
21-c POP – milk ko affect nahi karta.
22-b Warning signs (PAINS).
23-a Yaad aate hi lo.
24-d Cu-IUCD 5 din tak.
25-b Sahi.
26-b Ye to ideal candidate hai.
27-a 26 − 18 = 8; 32 − 11 = 21.
28-a Dono sahi, R reason hai.
29-c Tubectomy permanent.
30-b (3 × 1200) / 1200 = 3.

19 Gynaecological Disorders & Women's Health

Level 1 – Direct

1. Primary amenorrhoea ka sabse common chromosomal cause –
   (a) Turner's syndrome (45 XO) (b) Edward (c) Down's (d) Klinefelter
2. Secondary amenorrhoea me sabse pehle kya rule out karein?
   (a) TB (b) Tumour (c) PCOS (d) Pregnancy
3. Menorrhagia –
   (a) Cycle < 21 days (b) > 80 ml ya > 7 days, regular cycle (c) Scanty flow (d) Irregular bleeding
4. Polymenorrhoea –
   (a) No flow (b) Heavy flow (c) Cycle < 21 days (d) Cycle > 35 days
5. Primary dysmenorrhoea ka karan –
   (a) PID (b) Endometriosis (c) Prostaglandins (d) Fibroid
6. PCOS ka diagnostic criteria –
   (a) Rotterdam (2 of 3) (b) Bishop (c) Ballard (d) Amsel
7. Chocolate cyst –
   (a) Endometrioma (ovary me endometriosis) (b) Dermoid (c) Mole (d) Corpus luteum
8. Endometriosis ka gold standard diagnosis –
   (a) X-ray (b) Laparoscopy (c) Pap smear (d) Blood test
9. Infertility ki definition –
   (a) 1 year regular unprotected intercourse ke baad bhi conception nahi (b) 3 months (c) 5 years (d) 6 weeks
10. Infertility me pehla aur easiest test –
    (a) Laparoscopy (b) Semen analysis (c) HSG (d) Hysteroscopy
11. Sabse common benign uterine tumour –
    (a) Fibroid (leiomyoma) (b) Adenomyosis (c) Sarcoma (d) Polyp
12. Pregnancy me fibroid ki common degeneration –
    (a) Red degeneration (b) Sarcomatous (c) Hyaline (d) Calcific
13. Procidentia –
    (a) Pura uterus vulva se bahar (b) Cervix vagina me (c) Bladder prolapse (d) Rectocele
14. Bacterial vaginosis ki discharge –
    (a) Bloody (b) Frothy green (c) Thin grey, fishy odour, clue cells (d) Curdy white
15. Candidiasis discharge –
    (a) Fishy (b) Frothy (c) Watery (d) Thick, curdy white, intense itching
16. Strawberry cervix –
    (a) Gonorrhoea (b) Candida (c) Trichomoniasis (d) BV
17. India me women me sabse common cancer –
    (a) Cervix (b) Breast (c) Uterus (d) Ovary

18. Cervical cancer ka main cause –
(a) HIV (b) HPV 16 & 18 (c) HSV (d) EBV

? Level 2 – Priority & Application

19. Pap smear ke liye instruction –
(a) 48 hrs pehle douche/ intercourse/ vaginal medicine nahi, menses ke dauran nahi (b) Fasting (c) Full bladder (d) Enema

20. VIA me kaunsa solution use hota hai?
(a) Saline (b) KOH (c) Lugol's iodine (d) 3–5 % acetic acid

21. Breast self examination (BSE) ka best time –
(a) Menses ke dauran (b) Ovulation (c) Menses khatam hone ke 5–7 din baad, monthly (d) Kabhi bhi yearly

22. Post-menopausal bleeding wali 58 yr woman – sabse pehle rule out –
(a) Pregnancy (b) Endometrial carcinoma (c) Normal menses (d) Fibroid

23. Laparoscopy ke baad shoulder pain ka karan –
(a) Infection (b) MI (c) CO_2 gas se diaphragm irritation (referred pain) (d) Fracture

24. Hysterectomy post-op me nurse kya specifically monitor kare?
(a) Only diet (b) Breastfeeding (c) Only temperature (d) Vaginal bleeding, urine output (ureter injury), DVT signs

25. HPV vaccine (India) recommended age –
(a) Newborn (b) Girls 9–14 yrs (c) After menopause (d) 30 yrs

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Match: P. BV Q. Candida R. Trichomonas S. Chlamydia — with 1. Azithromycin 2. Clotrimazole 3. Metronidazole 4. Metronidazole both partners (STI)
(a) P-3, Q-4, R-2, S-1 (b) P-2, Q-3, R-4, S-1 (c) P-1, Q-2, R-4, S-3 (d) P-3, Q-2, R-4, S-1

27. Syndromic STI kits – vaginal discharge ke liye kit ka colour –
(a) Green (Kit 2) (b) Grey (Kit 1) (c) Yellow (Kit 6) (d) Red (Kit 5)

28. Assertion (A): HRT me uterus present ho to oestrogen ke saath progesterone diya jata hai. Reason (R): Unopposed oestrogen endometrial hyperplasia/ cancer ka risk badhata hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Menopause ka symptom NAHI hai –
(a) Vaginal dryness (b) Osteoporosis risk (c) Hot flushes (d) FSH kam ho jana

30. Uterovaginal prolapse ka risk factor NAHI hai –
(a) Nulliparity with regular exercise (b) Chronic cough (c) Menopause (d) Multiparity

✓ Answer Key & Explanation

1-a Turner's.
2-d Pregnancy.
3-b Menorrhagia.
4-c < 21 days.
5-c Prostaglandins.
6-a Rotterdam.
7-a Endometrioma.
8-b Laparoscopy.
9-a 1 yr (≥ 35 yrs – 6 months).
10-b Semen analysis.
11-a Fibroid.
12-a Red degeneration.

13-a Procidentia.
14-c BV.
15-d Candida.
16-c Trichomonas.
17-b Breast.
18-b HPV 16, 18.
19-a Pap prep.
20-d Acetic acid 3–5 %.
21-c 5–7 days after menses.
22-b Endometrial cancer.
23-c CO_2 referred pain.
24-d Bleeding, ureter, DVT.
25-b 9–14 yrs.
26-d Sahi.
27-a Green Kit 2.
28-a Dono sahi, R reason hai.
29-d Menopause me FSH badhta hai.
30-a Nulliparity protective.

20 GSSSB Pattern Mock Test – 100 MCQs (Full OBG Syllabus)



Mock test ke rules

100 questions · 100 marks · 75 minutes (OBG section ka practice). Galat answer par **−0.25**. Timer lagao, alag kagaz par answer likho, phir answer key se milao.

Score guide: 80+ Excellent · 65–79 Good · 50–64 Weak topics revise karo · < 50 Notes dobara padho.

? Mock Test – Part A (1–50)

1. MMR kis per express hota hai?
(a) 1000 population (b) 1000 live births (c) 1,00,000 live births (d) 10,000 births

2. PMSMA day –
(a) 15th (b) Last Sunday (c) 9th of every month (d) 1st

3. Para 0, gravida 1 woman –
(a) Grand multipara (b) Nulligravida (c) Multipara (d) Primigravida

4. Fertilisation site –
(a) Ampulla (b) Fundus (c) Isthmus (d) Cervix

5. Gynaecoid pelvis ka inlet –
(a) Kidney-shaped (b) Round (c) Heart-shaped (d) Oval AP

6. Obstetric conjugate –
(a) 8 cm (b) 10–10.5 cm (c) 12 cm (d) 13 cm

7. Posterior fontanelle closes –
(a) 18 months (b) 6–8 weeks (c) Birth (d) 1 year

8. Smallest engaging diameter (flexed vertex) –
(a) Suboccipito-bregmatic (b) Mento-vertical (c) Occipito-frontal (d) Submento-vertical

9. Ovulation ka trigger hormone –
(a) FSH fall (b) Prolactin (c) LH surge (d) Oxytocin

10. Organogenesis period –
(a) 0–2 wks (b) 3–8 wks (c) 12–16 wks (d) 20–24 wks

11. hCG peak –
(a) 20 wks (b) 30 wks (c) Term (d) 8–10 wks

12. Placenta weight –
(a) 1 kg (b) 1.5 kg (c) 250 g (d) 500 g

13. Umbilical vein carries –
(a) Lymph (b) Urine (c) Oxygenated blood to fetus (d) Deoxygenated blood to placenta

14. Oligohydramnios AFI –
(a) < 5 cm (b) 10 cm (c) 15 cm (d) > 25 cm

15. Ductus arteriosus becomes –
(a) Ligamentum teres (b) Ligamentum venosum (c) Ligamentum arteriosum (d) Fossa ovalis

16. Pregnancy me blood volume ↑ –
(a) 5 % (b) 10 % (c) 40–50 % (d) 100 %

17. Supine hypotension – position –
(a) Prone (b) Supine (c) Left lateral (d) Sitting with legs down

18. Chadwick's sign –
(a) Pulsation (b) Bluish discolouration of vagina (c) Cervix softening (d) Isthmus softening

19. Naegele's rule –
(a) LMP + 280 wks (b) LMP − 3 m − 7 d (c) LMP + 10 m (d) LMP + 9 m + 7 d

20. Positive sign of pregnancy –
(a) Nausea (b) Amenorrhoea (c) Quickening (felt by mother) (d) FHS heard

21. IFA prophylaxis in pregnancy –
(a) 100 mg × 100 days (b) 30 mg × 60 days (c) 60 mg Fe + 500 µg FA × 180 days (d) FA only

22. DIPSI GDM cut-off (2 hr) –
(a) ≥ 100 (b) ≥ 140 mg/dl (c) ≥ 180 (d) ≥ 200

23. Fundus at umbilicus –
(a) 12 wks (b) 16 wks (c) 22–24 wks (d) 36 wks

24. Pawlik's grip –
(a) Fundus (b) Liquor (c) Back (d) Presenting part

25. Normal FHR –
(a) 60–100 (b) 110–160 (c) 160–200 (d) 200+

26. Late deceleration –
(a) Head compression (b) Cord compression (c) Normal (d) Uteroplacental insufficiency

27. BPP parameters –
(a) 3 (b) 4 (c) 5 (d) 7

28. 1st stage of labour ends –
(a) Placenta (b) Baby birth (c) Engagement (d) Full cervical dilatation

29. Partograph alert line –
(a) 0.5 cm/hr (b) 1 cm/hr (c) 2 cm/hr (d) 3 cm/hr

30. Internal rotation occurs at –
(a) Inlet (b) Pelvic floor (c) Uterus (d) Outlet after delivery

31. AMTSL uterotonic –
(a) Methergine IV bolus (b) Oxytocin 10 IU IM (c) Carboprost (d) Misoprostol 25 µg

32. Sign of placental separation –
(a) Uterus becomes soft (b) Fever (c) Bradycardia (d) Cord lengthening

33. 3rd degree tear involves –
(a) Rectal mucosa (b) Skin only (c) Anal sphincter (d) Vaginal mucosa only

34. Lochia serosa –
(a) Day 5–9 (b) Day 1–4 (c) Day 10–6 wks (d) Day 42

35. Uterus not palpable abdominally by –
(a) 6 months (b) 10–14 days (c) 3 days (d) 6 weeks

36. Milk ejection reflex hormone –
(a) Oxytocin (b) Prolactin (c) Progesterone (d) Oestrogen

37. Exclusive breastfeeding –
(a) 1 year (b) 2 years (c) 3 months (d) 6 months

38. Mastitis – breastfeeding –
(a) Continue (b) Stop (c) Formula only (d) Only other side forever

39. APGAR "G" –
(a) Glucose (b) Grimace (reflex irritability) (c) Growth (d) Gestation

40. Neonatal CPR ratio –
(a) 3 : 1 (b) 5 : 1 (c) 15 : 2 (d) 30 : 2

41. Vitamin K dose (term newborn) –
(a) 0.1 mg (b) 1 mg IM (c) 5 mg (d) 10 mg

42. Physiological jaundice appears –
(a) After 3 wks (b) After 24 hrs (c) Within 24 hrs (d) At birth

43. KMC is best for –
(a) Mothers only (b) Stable LBW babies (c) Sick ventilated babies (d) Post-term only

44. Anaemia in pregnancy (WHO) –
(a) Hb < 13 (b) Hb < 9 (c) Hb < 7 only (d) Hb < 11 g/dL

45. Eclampsia drug of choice –
(a) Diazepam (b) $MgSO_4$ (c) Phenytoin (d) Labetalol

46. $MgSO_4$ antidote –
(a) Atropine (b) Protamine (c) Calcium gluconate (d) Naloxone

47. GDM drug of choice –
(a) Metformin only (b) Acarbose (c) Insulin (d) Glibenclamide

48. Anti-D after delivery –
(a) Not needed (b) 300 µg within 72 hrs (c) 1000 µg IV (d) 50 µg after 7 days

49. Spalding's sign –
(a) Ectopic (b) Mole (c) IUD – overlapping skull bones (d) Twins

50. Heart disease in pregnancy – drug avoided in 3rd stage –
(a) Methergine (b) Morphine (c) Frusemide (d) Oxytocin

? Mock Test – Part B (51–100)

51. Commonest cause of 1st trimester abortion –
(a) Fibroid (b) Trauma (c) Cervical incompetence (d) Chromosomal anomaly

52. Threatened abortion – os –
(a) Products passed (b) Closed (c) Absent (d) Open

53. MTP Act 2021 – up to 20 wks –
(a) Court order (b) Medical board (c) 1 RMP (d) 2 RMPs

54. Ectopic commonest site –
(a) Cervix (b) Ovary (c) Ampulla (d) Interstitial

55. Medical management of ectopic –
(a) Methotrexate (b) Heparin (c) Oxytocin (d) Ergometrine

56. Snowstorm appearance –
(a) Twins (b) Ectopic (c) IUD (d) H. mole

57. Painless recurrent APH –
(a) Cervicitis (b) Abruption (c) Rupture (d) Placenta praevia

58. Couvelaire uterus –
(a) Inversion (b) Mole (c) Abruptio placentae (d) Praevia

59. In APH – contraindicated –
(a) PV examination (b) IV line (c) FHS (d) Cross-match

60. Commonest malpresentation –
(a) Brow (b) Breech (c) Face (d) Shoulder

61. After-coming head of breech –
(a) Woods (b) Zavanelli (c) MSV manoeuvre (d) McRoberts

62. Brow presentation – delivery –
(a) Spontaneous (b) Vacuum (c) Forceps (d) LSCS

63. Bandl's ring –
(a) Preterm (b) Abortion (c) Normal labour (d) Obstructed labour

64. Post-term pregnancy –
(a) ≥ 38 wks (b) ≥ 40 wks (c) ≥ 42 wks (d) ≥ 44 wks

65. Favourable Bishop score –
(a) 0 (b) 2 (c) ≤ 4 (d) ≥ 8

66. Preterm labour tocolytic first-line –
(a) Methergine (b) Misoprostol (c) Oxytocin (d) Nifedipine

67. Antenatal steroid –
(a) Hydrocortisone 100 mg × 1 (b) Prednisolone 5 mg (c) Dexamethasone 6 mg IM 12-hrly × 4 (d) Insulin

68. PPH (vaginal) –
(a) ≥ 100 ml (b) ≥ 300 ml (c) ≥ 500 ml (d) ≥ 2000 ml

69. Commonest cause of PPH –
(a) Inversion (b) Coagulopathy (c) Atony (d) Trauma

70. First step in atonic PPH –
(a) Hysterectomy (b) B-Lynch (c) Call for help + uterine massage (d) Wait

71. Carboprost contraindicated –
(a) Hypertension (b) Anaemia (c) Diabetes (d) Asthma

72. Uterine inversion – shock type –
(a) Anaphylactic (b) Cardiogenic (c) Septic (d) Neurogenic

73. Cord prolapse – position –
(a) Knee-chest (b) Lithotomy (c) Supine (d) Fowler's

74. Shoulder dystocia – avoid –
(a) Fundal pressure (b) Episiotomy evaluation (c) McRoberts (d) Suprapubic pressure

75. DIC commonest obstetric cause –
(a) Abruption (b) Hyperemesis (c) Breech (d) Praevia

76. Puerperal pyrexia –
(a) ≥ 36 °C (b) ≥ 37 °C (c) ≥ 38 °C (d) ≥ 40 °C

77. DVT postnatal – NOT to do –
(a) Massage the leg (b) Inform doctor (c) Anticoagulant (d) Elevate leg

78. Puerperal psychosis –
(a) Only physical (b) Self-limiting in 3 days (c) Psychiatric emergency (d) Normal blues

79. Sheehan's first sign –
(a) Lactation failure (b) Hypertension (c) Weight gain (d) Polyuria

80. Erb's palsy –
(a) Facial nerve (b) Radial (c) C8–T1 – claw hand (d) C5–C6 – waiter's tip

81. Commonest CS type –
(a) Classical (b) LSCS (c) Extraperitoneal (d) Inverted T

82. VBAC contraindicated after –
(a) Vaginal delivery (b) Forceps (c) One LSCS (d) Classical CS

83. Mendelson's syndrome –
(a) Hypotension (b) Acid aspiration under GA (c) Uterine atony (d) Spinal headache

84. Vacuum contraindicated –
(a) Prolonged 2nd stage vertex (b) Fetal distress 2nd stage, engaged vertex (c) Maternal exhaustion (d) Face presentation

85. Pethidine – neonatal depression antidote –
(a) Dopamine (b) Naloxone (c) Adrenaline (d) Atropine

86. Epidural commonest side effect –
(a) Seizure (b) Hyperglycaemia (c) Hypertension (d) Hypotension

87. Methergine contraindicated –
(a) Diabetes (b) Hypertension (c) Asthma (d) Hypothyroid

88. Heat-stable uterotonic –
(a) Carboprost (b) Misoprostol (c) Oxytocin (d) Methergine

89. Safe anticoagulant in pregnancy –
(a) Heparin (b) Warfarin (c) Rivaroxaban (d) Phenindione

90. Gray baby syndrome –
(a) Chloramphenicol (b) Gentamicin (c) Penicillin (d) Tetracycline

91. Cu-T 380A life –
(a) 1 yr (b) 3 yrs (c) 5 yrs (d) 10 yrs

92. Chhaya –
(a) Monthly injection (b) Implant (c) Non-steroidal weekly pill (d) Combined daily pill

93. Antara –
(a) IUCD (b) Weekly pill (c) DMPA 150 mg 3-monthly (d) Condom

94. Emergency contraception LNG –
(a) 0.03 mg daily (b) 1.5 mg within 72 hrs (c) 5 mg after 7 days (d) 10 mg

95. LAM – criteria NOT included –
(a) Exclusive breastfeeding (b) Amenorrhoea (c) Baby < 6 months (d) Baby > 1 year

96. Fishy odour with clue cells –
(a) Bacterial vaginosis (b) Trichomonas (c) Candida (d) Gonorrhoea

97. Strawberry cervix –
(a) Trichomoniasis (b) Candida (c) BV (d) Chlamydia

98. Cervical cancer screening (VIA) –

(a) 3–5 % acetic acid (b) Saline (c) KOH (d) Lugol's iodine

99. Commonest cancer in Indian women –

(a) Breast (b) Ovary (c) Endometrium (d) Cervix

100. Menopause hormonal marker –

(a) ↑ Progesterone (b) ↑ FSH (c) ↓ FSH (d) ↑ Oestrogen

Answer Key – Mock Test (with explanation)

1-c Per 1,00,000 live births.
2-c 9th of every month.
3-d Primigravida.
4-a Ampulla.
5-b Round.
6-b 10–10.5 cm.
7-b 6–8 weeks.
8-a SOB 9.5 cm.
9-c LH surge.
10-b 3–8 wks.
11-d 8–10 wks.
12-d 500 g.
13-c Oxygenated to fetus.
14-a AFI < 5.
15-c Ligamentum arteriosum.
16-c 40–50 %.
17-c Left lateral.
18-b Bluish vagina.
19-d LMP + 9 m + 7 d.
20-d FHS – positive sign.
21-c 60 mg + 500 µg × 180 days.
22-b ≥ 140.
23-c 22–24 wks.
24-d Presenting part.
25-b 110–160 bpm.
26-d Placental insufficiency.
27-c 5.
28-d Full dilatation.
29-b 1 cm/hr.
30-b Pelvic floor.
31-b Oxytocin 10 IU IM.
32-d Cord lengthening.
33-c Anal sphincter.
34-a Day 5–9.
35-b 10–14 days.
36-a Oxytocin.
37-d 6 months.
38-a Continue.
39-b Grimace.
40-a 3 : 1.
41-b 1 mg IM.
42-b After 24 hrs.
43-b Stable LBW.
44-d Hb < 11.
45-b $MgSO_4$.
46-c Calcium gluconate.
47-c Insulin.
48-b 300 µg within 72 hrs.
49-c IUD.
50-a Methergine.
51-d Chromosomal.
52-b Closed.
53-c 1 RMP.
54-c Ampulla.
55-a Methotrexate.
56-d Mole.
57-d Praevia.
58-c Abruption.
59-a PV exam.
60-b Breech.
61-c MSV.
62-d LSCS.
63-d Obstructed labour.
64-c ≥ 42 wks.
65-d ≥ 8.
66-d Nifedipine.
67-c Dexamethasone 6 mg × 4.
68-c ≥ 500 ml.
69-c Atony.
70-c Help + massage.
71-d Asthma.
72-d Neurogenic.
73-a Knee-chest.
74-a Fundal pressure.
75-a Abruption.
76-c ≥ 38 °C.
77-a Massage – embolism risk.
78-c Emergency.
79-a Lactation failure.
80-d C5–C6.
81-b LSCS.
82-d Classical.
83-b Acid aspiration.
84-d Face – vacuum contraindicated.
85-b Naloxone.
86-d Hypotension.
87-b Hypertension.
88-b Misoprostol.

89-a Heparin.
90-a Chloramphenicol.
91-d 10 yrs.
92-c Weekly non-steroidal.
93-c DMPA.
94-b 1.5 mg in 72 hrs.
95-d Baby > 1 yr – LAM invalid.
96-a BV.
97-a Trichomonas.
98-a Acetic acid.
99-a Breast.
100-b ↑ FSH.

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